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Date

04/10/2026<https://www.smartrequest.com/>**Ship To:**

Dallas Flaherty
Flaherty, Dallas
61 DRAKE AVE
STOUGHTON,MA 02072-2236

Requested By: FLAHERTY, DALLAS**Patient Name:** FLAHERTY DALLAS**DOB :** 11/03/1994**Records From:**

BOSTON MEDICAL CENTER SOUTH
235 NORTH PEARL ST
BROCKTON,MA 02301

Please Note:

Your medical record request has been
delivered electronically to your Smartrequest
account.

0553279578

INVOICE NUMBER

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AUTHORIZATION FOR RELEASE OF PROTECTED HEALTH INFORMATION (PHI)

Mailing Address:

Medical Record Department
850 Harrison Avenue/ACC Basement
Boston, MA 02118

Fax: 617-414-4210
Phone: 617-414-4213

Patient Name: Flaherty		Dallas	G
Last		First	MI
Address: 61 Drake Ave			
Street (include Apt #, if applicable)			
Stoughton		MA	02072
City		State	Zip Code
Birth Date: 11/03/1994		Telephone #: (781) 588-2221	MR#: _____
ALTERNATE ADDRESS: (Please indicate if the information is to be sent to a different address, that is other than the address listed above).			
Street (include Apt #, if applicable)			
City		State	Zip Code
I hereby authorize Boston Medical Center to <u>release</u> my protected health information to (choose one method per request):			
☐ Mail to: _____		☐ Hold for pickup by: _____	
☐ Fax (only to healthcare facilities): _____		☒ Email: dallasgflaherty@gmail.com	
Name: Dallas Flaherty			
Address: 61 Drake Ave, Stoughton, MA 02072			

PLEASE CHECK THE FORMAT YOU PREFER TO RECEIVE YOUR MEDICAL RECORDS: ☐ PAPER ☒ ELECTRONIC

PURPOSE OF DISCLOSURE (Please check one):

☒ Myself ☐ Inspection ☐ Changing physicians ☐ Consultation ☐ School ☐ Legal ☐ Other (specify): _____

INFORMATION TO BE RELEASED (Please be specific and enter dates of service, date range and/or clinic names):

- ☐ Medical Record Abstract (e.g., ED, H&P, Operative Rpt, Discharge Summary, Consults, Labs, X-rays, Pathology): _____
- ☐ Clinic Notes _____ ☒ Pathology Reports _____
- ☒ Consultation Reports _____ ☒ MRI Reports _____
- ☒ Medication Records _____ ☒ ED Record _____
- ☐ Complete Records _____ ☒ Other (specify content) _____

Admission Sheet, Itemized Billing Statements, Discharge Summary, Cardiology W/EKG Report, History / Physical Report, Immunization Report, Laboratory Report, Operative / Procedure Report, Physician Orders Report, Nurse Notes, Progress Notes, Therapy Notes

TO REQUEST THE RELEASE OF SPECIFICALLY PROTECTED OR PRIVILEGED INFORMATION, YOU MUST INITIAL BELOW:

- D.F. HIV Test Results (PATIENT AUTHORIZATION REQUIRED FOR EACH RELEASE REQUEST).
- D.F. Sexually Transmitted Disease (STDs) D.F. Genetic Counseling
- D.F. Commonwealth of Massachusetts Sexual Assault D.F. Domestic Violence
- D.F. Evidence Collection Kit/Sexual Assault Counseling D.F. Social Work Counseling/Therapy
- D.F. Psychiatric Records or Information D.F. Professional services of a licensed psychologist
- D.F. Psychotherapy Notes (Notes recorded by a mental health professional documenting or analyzing the contents of a conversation, during a private counseling session or group, joint, family counseling, and that are separate from the medical record).
- D.F. Alcohol and Drug Abuse Records Protected by Federal Confidentiality Rules 42 CFR Part 2.
- FEDERAL RULES PROHIBIT ANY FURTHER DISCLOSURE OF THIS INFORMATION UNLESS DISCLOSURE IS EXPRESSLY PERMITTED BY WRITTEN AUTHORIZATION OF THE PERSON TO WHOM IT PERTAINS, OR AS OTHERWISE PERMITTED BY 42 CFR PART 2.

I understand that I have the right to withdraw my authorization at any time except to the extent that action has been taken on reliance on this authorization. I understand that if I revoke this authorization, I must do so in writing and present my written revocation to the Director of Medical Records. I understand that authorizing the disclosure of this health information is voluntary, I can refuse to sign, and Boston Medical Center will not condition my treatment, payment, health plan enrollment, or eligibility for benefits on my providing authorization for the requested use or disclosure. *I understand that health information used or disclosed pursuant to this authorization may be subject to redisclosure by the recipient, and no longer protected by Federal Confidentiality regulations; however the recipient may be prohibited from disclosing substance abuse information.* I understand that I may inspect or copy the information to be disclosed, for a reasonable charge.

If I fail to specify an expiration date or event, and unless otherwise revoked, this authorization will expire **six months** from the date of the signature listed below. I have carefully read and understand the above, have had any questions explained to my satisfaction, and do herein expressly and voluntarily authorize disclosure of the above information about, or medical records of my condition to those persons or agencies listed above.

Signature of Patient (18 years or older) Date **04/02/2026**

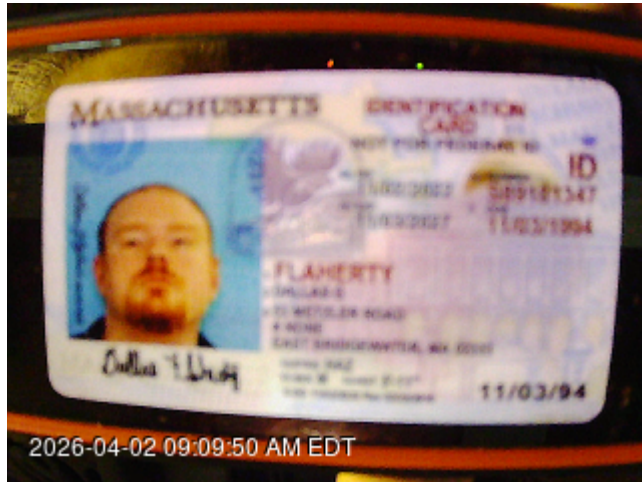
Signature of Legal Representative _____ **Relationship to Patient:** _____ **Date** _____

Please make a copy of this release for your records.

Dallas Flaherty (D.O.B. 11/03/1994)

Additional Identifiers Requested

1. Government ID



2. Mailing Address

61 Drake Ave
Stoughton, MA 02072

3. Email Address (user confirmed)

dallasgflaherty@gmail.com (consented to an emailed copy of their request)

4. Phone Number (verified)

(781) 588-2221 ✓

5. Requestor Signature

Handwritten signature of Dallas Flaherty.

REASON FOR REQUEST

Patient Request

REQUEST SPECIFIC COMMENTS

Please send all records related to my recent stay. thank you

SENT	04/02/2026 09:09:49 GMT-04:00	Sent for signature to Dallas Flaherty (dallasgflaherty@gmail.com)
VIEWED	04/02/2026 09:09:51 GMT-04:00	Viewed by Dallas Flaherty (dallasgflaherty@gmail.com) IP: 24.91.130.109
SIGNED	04/02/2026 09:11:10 GMT-04:00	Signed by Dallas Flaherty (dallasgflaherty@gmail.com) IP: 24.91.130.109
COMPLETED	04/02/2026 09:11:12 GMT-04:00	The document has been completed. (Request ID: KHWJGZ8I)



BMC Yawkey
850 Harrison Ave.
Boston MA02118-4001

Flaherty, Dallas
MRN:3394094
DOB:11/3/1994,Sex:male
SSN:022-78-8206
Visit Date:

Patient

Demographics

Name: Dallas Flaherty
Address: 61 Drake Ave Stoughton MA 02072-2236
Date of birth: 11/3/1994 Legal sex: Male Gender identity: Male
Ethnicity: Not Hispanic, Latino, Latina, Latinx, Race: White Language: English
or of Spanish or Latin American origin
Mobile: 781-588-2221

Relationships

Name	Relation to Patient	Phone Number
Flaherty, Terry	Other Relationship	Mobile: 781-815-8001 (primary)

Immunizations

No documentation.



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Visit Date:2/18/2026

02/18/2026 - Admission (Discharged) in Brockton 3 Behavioral Health Inpatient Unit

Notes

Discharge Summary by Jared K Trout, MD at 2/24/2026 1117

Inpatient PSY Discharge Summary

Admission Date: 2/18/2026 11:19 PM

Discharge Date: 2/24/26

Admitting Provider: Ajay Mehta, MD

Discharge Provider: No att. providers found

Admitting Diagnosis/Reason for Admission:

MANIA

Discharge Diagnoses:

Principal Problem:

Bipolar disorder with depression (CMS/HHS/HCC)

Active Problems:

Cannabis use disorder

Treatment, Response, Hospital Course: Mr. Flaherty is a 31 year-old man BIBA on a sec 12 after his mother called the police reporting that pt kicked in her door, was yelling at her and threatening to kill her. Pt's mother states he woke up "angry" today. Pt denies HI/AH/VH, reports passive SI, feeling so overwhelmed "I can't take it anymore".

He presents after a high-risk domestic incident involving threats to kill his mother and a reported knife threat. Even though he minimizes intent and describes it as accidental and sheathed, the reported behavior represents significant acute risk given escalation, impulsivity, agitation, and access to a weapon during conflict. His presentation is characterized by marked irritability, hyperverbal speech, severe anxiety, poor sleep, appetite reduction, and impaired behavioral control. He denies current psychotic symptoms and denies current suicidal ideation. He has a history of a suicide attempt, chronic psychosocial stressors, and limited supports.

Agitated, irritable, labile, threatening, demanding to be discharged. He is showing marked irritable and manic symptoms. He insisted on being discharged immediately and was becoming quite agitated but was with effort able to be redirected by staff and accepted olanzapine 10 mg PO and lorazepam 2 mg PO. He denies HI but still appears quite angry with his mother, although at the same time hoping she sill pick him up and bring him home. We will increased



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02/18/2026 - Admission (Discharged) in Brockton 3 Behaviorall Health Inpatient Unit (continued)

Notes (continued)

olanzapine to 10 mg at bedtime. We will add lorazepam as needed for anxiety and manic symptoms. We will consolidate depakote to 1500 mg daily. We will follow up depakote level tomorrow. We will get collateral as able. We will provides structure and support. We will continue to observe.

Three day letter expires on 2/23/26.

PSYCHIATRIC HISTORY (E.g, past diagnoses, hospitalizations, self-harm)

He reports diagnoses of PTSD and bipolar disorder with at least one prior psychiatric hospitalization, including treatment at Westborough Hospital. He reports a past suicide attempt in 2014 by attempted strangulation with a belt. He denies recent self-injurious behavior. He denies current outpatient therapy and reports only psychiatric medication management.

SUBSTANCE / ALCOHOL / TOBACCO USE HISTORY

He reports daily/regular cannabis use and intermittent alcohol use. He denies cocaine and opioids. Urine toxicology is positive for cannabinoids. He reports tobacco use with current cravings.

SOCIAL / TRAUMA / DEVELOPMENTAL HISTORY (E.g, employment, housing, interpersonal supports)

Pt and his mother report pt was born with the cord wrapped around his neck 4 times, 0 APGAR and required brain surgery/a shunt. Pt's mother reports this led to some developmental/cognitive delays and pt has also struggled with mental health issues throughout his life. Pt reports he grew up in the Bridgewater area, he has 3 older sisters who he "hates". Pt reports he and his mother "butt heads all the time..she does nothing to help me". Pt's mother reports he has a lot of close cousins his age and has struggled to see them get married/have kids/hold jobs/drive while he has not been able to do any of that.

Mr. Flaherty was initially agitated, labile, intense, tearful, demanding to leave the hospital immediately. Yet he was able to remain in gross behavioral control, accepted medications, and agreed to cooperate with the treatment team. We gave olanzapine for manic symptoms and mood stabilization and titrated to olanzapine 10 mg BID. We also resumed depakote ER 1500 mg daily. He had reproted a depakote level in the 100s as an outpatient and is following up with his outpatient psychiatrist. Depakote level was 74.9 by our measurement on 2/20/26 on a dose of depakote ER 1500 mg daily, though he will want to follow up with his outpatient psychiatrist as well.

Mr Flaherty responded well to this regimen. He became more calm and rational and less impulsive. He consistently denied SI/HI. He agreed to outpatient care. He signed a three day letter expiring on 2/23/26. This was the day of the Blizzard of 2026 and he rationally agreed to revoke his three day letter and sign another three day letter with the understanding that he would be discharged the next day.



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02/18/2026 - Admission (Discharged) in Brockton 3 Behavioral Health Inpatient Unit (continued)

Notes (continued)

On day of discharge (2/24/26), Mr. Flaherty was pleasant and engaging. He said that things were "Going pretty well." He denied SI/HI and AVH. He said "I need to go home." He appeared much improved from presentation. Although there was still some intensity of mood he was more calm and in better behavioral control. He willingness to retract his three day letter the previous day also showed his restraint and rationality. He said that he was looking forward to shoveling snow at home. He did not appear acutely manic or psychotic. He did not appear to be a threat to himself or others. Overall, we are comfortable discharging him with strong family and outpatient support. Protective factors include strong family support (including from his mother), stable place to stay, DDS services, referral for BAMSI Adult Day Treatment, outpatient appointments, medication regimen, and positive attitude.

We discussed side effects of depakote such as weight gain.

We discussed side effects of olanzapine such as weight gain. He was also advised that tobacco smoking can lower the concentration of olanzapine and render the medicine less effective.

We also counseled against cannabis use.

There was a ban on nonessential travel by executive order of the governor in Plymouth County at this time. Road not completely plowed and there was a possible \$500 fine for nonessential travel. He said that his mother lives in Stoughton and would come drive to the hospital and pick him up. I call his mother and informed her of the travel band, the risk of using the roads, and the possibility of the \$500 fine. She acknowledged this and said she still wanted to come and pick up her son. Accordingly, she arrived at the hospital and Mr. Flaherty was discharged and she was avised to drive safely.

Medical Issues Addressed During Hospitalization:

Labs (see Results Review Activity for complete result history):

No results found for this visit on 02/18/26 (from the past 96 hours).

Mental Status Exam:

General Appearance: well groomed

Behavior: Cooperative

Mood(describe in patient's own words): "Going pretty well."

Affect: Euthymic and Full Range



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02/18/2026 - Admission (Discharged) in Brockton 3 Behavioral Health Inpatient Unit (continued)

Notes (continued)

Speech: fluent, normal rate, normal tone, and normal volume

Relevance: relevant

Thought Process: Coherent and Goal Directed

Thought Content

Perception: Normal

Delusions: None

Suicidal Ideation: No

Violent Ideation: No

C-SSRS DISCHARGE SCREENER (will automatically populate if completed)

C-SSRS Discharge Screener

- 1) Wish to be Dead: While you were here in the program, have you wished you were dead or wished you could go to sleep and not wake up?: No
- 2) Suicidal Thoughts: While you were here in the program, have you actually had thoughts about killing yourself?: No
- 6) Suicide Behavior: While you were here in the program, have you done anything, started to do anything, or prepared to do anything to end your life?: No

Discharge Suicide Assessment and Safety Intervention:

Formulation of Suicide Risk at Discharge (Explain Changes in Clinical Status, Risk Factors, and Risk Mitigation): Suicidal ideation was not part of the patient's presenting complaint. He consistently denied SI on the inpatient unit. There were no incidents of SIB on the inpatient unit. He was cooperative with the team and improved markedly with treatment. He was less impulsive and more organized and rational. He was looking forward to going home and following up with his care. We were also able to make a referral for adult day treatment. On day of discharge (2/24/26), Mr. Flaherty was pleasant and engaging. He said that things were "Going pretty well." He denied SI/HI and AVH. He said "I need to go home." He appeared much improved from presentation. Although there was still some intensity of mood he was more calm and in better behavioral control. He willingness to retract his three day letter the previous day also showed his restraint and rationality. He said that he was looking forward to shoveling snow at home. He did not appear acutely manic or psychotic. He did not appear to be a threat to himself or others. On day of discharge (2/24/26), Mr. Flaherty was pleasant and engaging. He said that things were "Going pretty well." He denied SI/HI and AVH. He said "I need to go home." He appeared much improved from presentation. Although there was still some intensity of mood he was more calm and in better behavioral control. He willingness to retract his three day letter the previous day also showed his restraint and rationality. He said that he was looking forward to shoveling snow at home. He did not appear acutely manic or psychotic. He did not appear to be a threat to himself or others. For patients who expressed suicidal thoughts or engaged in suicidal behaviors prior to admission or during hospitalization, have factors contributing to patient's suicidal ideas or behaviors improved during hospitalization? No history of suicidal thoughts or behavior
Firearms, if identified, have been disposed of, or efforts made to accomplish this? N/A Information about hot lines and other crisis information provided to patient? Yes



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02/18/2026 - Admission (Discharged) in Brockton 3 Behaviorall Health Inpatient Unit (continued)

Notes (continued)

Safety Plan: Yes,

Homicide Interventions: He came to the hospital after yelling at his mother with whom he lives, threatening to kill her, and brandishing a knife. Yet he consistently denied HI on the inpatient unit. He admitted that he should not have threatened his mother or picked up the knife and noted that the knife remained sheathed during the incident. He consistently denied HI or thought of hurting his mother. Although he was initially irritable and agitated on the unit and demanding to leave, he was able to maintain behavioral control and accept treatment. There were no incidents of violence on the inpatient unit. He improved markedly with treatment and was less intense and labile and more organized and rational. He was even rational enough and showed enough restraint and capacity for delayed gratification as to retract his three day letter in order to wait out the blizzard and avoid dangerous travel on that day. Collateral from his mother was supportive of discharge and she agreed to pick him up from the hospital.

Prognosis: good

Discharge Type: 3-Day Notice

Discharge To: private house/apartment

Active Issues Requiring Follow-up

Test Results Pending at Discharge

Follow-Up Care

Appointment Instructions:

Discharge References/Attachments

BMC ED THRIVE SDOH REFERRAL HANDOUT (ARABIC)

Discharge Medications

Your medication list

START taking these medications



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02/18/2026 - Admission (Discharged) in Brockton 3 Behaviorall Health Inpatient Unit (continued)

Notes (continued)

	Details	Instruction
loratadine 10 mg tablet Commonly known as: CLARITIN	Dose: 10 mg Quantity: 30 tablet Refills: 0	Take 1 tablet (10 mg total) by mouth daily for 30 days.
nicotine polacrilex 2 mg gum Commonly known as: NICORETTE	Dose: 2 mg Quantity: 100 each Refills: 0	Take 1 each (2 mg total) by mouth every 2 (two) hours as needed for smoking cessation. Chew gum and then place between cheek and gum.
pantoprazole 40 mg EC tablet Commonly known as: PROTONIX Replaces: omeprazole 20 mg capsule	Dose: 40 mg Quantity: 30 tablet Refills: 0	Take 1 tablet (40 mg total) by mouth daily for 30 days. Swallow whole; do NOT crush, split, or chew.

CHANGED/REFILLED medications

	Details	Instruction
divalproex 500 MG 24 hr tablet What changed: • how much to take • when to take this	Dose: 1,500 mg Quantity: 90 tablet Refills: 0	Take 3 tablets (1,500 mg total) by mouth daily.



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Notes (continued)

	Details	Instruction
melatonin 3 mg Tab	Dose: 9 mg	Take 3 tablets (9 mg
What changed:	Quantity: 90 tablet	total) by mouth nightly
• medication strength • how much to take • when to take this • reasons to take this	Refills: 0	for 30 days.

OLANzapine 10 MG tablet	Dose: 10 mg	Take 1 tablet (10 mg
Commonly known as: ZyPREXA	Quantity: 60 tablet	total) by mouth 2 (two)
What changed:	Refills: 0	times a day for 30 days.
• medication strength • how much to take • when to take this		

CONTINUE taking these medications

	Details	Instruction
amLODIPine 10 mg tablet	Dose: 10 mg	Take 1 tablet (10 mg
Commonly known as: NORVASC	Quantity: 30 tablet	total) by mouth daily.
	Refills: 0	
busPIRone 10 mg tablet	Dose: 10 mg	Take 1 tablet (10 mg
Commonly known as: BUSPAR	Quantity: 60 tablet	total) by mouth 2 (two)
	Refills: 0	times a day.
hydroCHLORothiazide 25 mg tablet	Dose: 25 mg	Take 1 tablet (25 mg
Commonly known as: HYDRODIURIL	Quantity: 30 tablet	total) by mouth daily for
	Refills: 0	30 days.



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02/18/2026 - Admission (Discharged) in Brockton 3 Behavioral Health Inpatient Unit (continued)

Notes (continued)

STOP taking these medications

omeprazole 20 mg capsule

Commonly known as: PriLOSEC

Replaced by: **pantoprazole 40 mg EC tablet**

Where to Get Your Medications

These medications were sent to BMC Brockton
Behavioral Health Center - Brockton, MA - 34 N.
Pearl St.

34 N. Pearl St., Brockton MA 02301

Phone: 617-638-9320

- amLODIPine 10 mg tablet
- busPIRone 10 mg tablet
- divalproex 500 MG 24 hr tablet
- hydroCHLOROthiazide 25 mg tablet
- loratadine 10 mg tablet
- melatonin 3 mg Tab
- nicotine polacrilex 2 mg gum
- OLANZapine 10 MG tablet
- pantoprazole 40 mg EC tablet

Physical Condition at Discharge



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02/18/2026 - Admission (Discharged) in Brockton 3 Behaviorall Health Inpatient Unit (continued)

Notes (continued)

Physical Condition Upon Discharge: good

Has Patient Used Tobacco During the Past 30 Days? Yes, Yes

has no history on file for tobacco use.

Is the Patient Discharged on More than One Anti-Psychotic Medication? no

Activity: no restrictions

Electronic Signature:

Jared K Trout, MD

2/24/2026 7:06 PM

Electronically signed by Jared K Trout, MD at 2/25/2026 7:48 PM

Discharge Summary by Nicole Giardino, LICSW at 2/24/2026 0956

SOCIAL WORK DISCHARGE SUMMARY

Diagnosis Upon Discharge:

Principal Problem:

Bipolar disorder with depression

Active Problems:

Cannabis use disorder (per record)

Admission Date: 2/19/2026

Discharge Date: 2/24/2026

TREATMENT COURSE SUMMARY:

The patient was admitted to inpatient psychiatric level of care/hospital at BMC Brockton Behavioral Health Center due to concerns of the patient presenting as aggressive, threatening to kill his mother and reporting passive SI. During the patient's admission here, per the inpatient provider, "....On day of discharge (2/24/26), Mr. Flaherty was pleasant and engaging. He said that things were "Going pretty well." He denied SI/HI and AVH. He said "I need to go home." He appeared much improved from presentation. Although there was still some intensity of mood he was more calm and in better behavioral control. He willingness to retract his three day letter the previous day also showed his restraint and rationality. He said that he was looking forward to shoveling snow at home. He did not appear acutely manic or psychotic. He did not appear to be a threat to himself or others. Overall, we are comfortable discharging him with strong family and outpatient support. Protective factors include strong family support (including from his mother), stable place to stay, DDS services, referral for BAMSI Adult Day Treatment, outpatient appointments, medication regimen, and positive attitude. We discussed side effects of depakote such as weight gain. We discussed side effects



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02/18/2026 - Admission (Discharged) in Brockton 3 Behaviorall Health Inpatient Unit (continued)

Notes (continued)

of olanzapine such as weight gain. He was also advised that tobacco smoking can lower the concentration of olanzapine and render the medicine less effective. We also counseled against cannabis use. There was a ban on nonessential travel by executive order of the governor in Plymouth County at this time. Road not completely plowed and there was a possible \$500 fine for nonessential travel. He said that his mother lives in Stoughton and would come drive to the hospital and pick him up. I call his mother and informed her of the travel band, the risk of using the roads, and the possibility of the \$500 fine. She acknowledged this and said she still wanted to come and pick up her son. Accordingly, she arrived at the hospital and Mr. Flaherty was discharged and she was avised to drive safely." During the patient's admission here, with this writer, the patient engaged in case management, limited willingness to attend multiple offered group therapy sessions, ensured connection to community resources, discussing coping skills and setting up aftercare appointments. The patient reports planning to discharge to and reside with his mother in Stoughton, MA. During the patient's admission, this writer contacted collaterals of Terry Flaherty (mother), Ricky Stebbins (advocate) and Christine (DDS/Baycove case worker - verbal consent obtained only) during the hospitalization, at the pt's request. Patient's mother denied any safety concerns at the time of discharge and expressed comfort with the current discharge and aftercare plan moving forward.

The patient was provided a patient survey. Writer met with pt in a private room on the unit to discuss the pt's discharge plan, alongside LIP, Dr. Trout. LIP explained that pt's Depakote level was a bit higher than prior. Pt reported struggling in this environment, feeling stable and safe, to be discharged. Pt reported having fair appetite and reported impaired sleep the night prior. LIP explained confirming the d/c with pt's mother and reported she plans to pick him up; (of note, LIP informed pt's mother of the current travel ban due to the snow storm, all fines and risks involved with currently traveling.) Pt denied any further concerns. Pt denied any current suicidal ideation and acknowledged contributing factors to this admission. Pt expressed a willingness and desire to continue to receive treatment and to abstain from illicit substance use. Pt completed and review a safety plan with this writer, of which the pt expressed understanding of the pt's safety plan. Pt denied having any access to weapons or firearms, stockpiled medications, or any lethal means to end pt's life or someone else's in pt's residence or the wider community. Writer provided pt with local crisis # in the region the pt will be living in. Patient was strongly recommended to call 911 or go to nearest emergency room or call suicide hotline in case of any acute medical or psychiatric crisis. Pt reported feeling safe about leaving the facility. Pt continues to be in agreement with this plan and denies any questions at this time.

Pt presented as alert and oriented, fair in appearance, normal eye contact, speech normal rate and rhythm, appropriate in mood with congruent affect, denies suicidal and homicidal ideation, denies auditory and visual hallucinations, no overt indication of psychosis evident, insight, judgment, and impulsivity appear to be within normal limits.

Pt discharged to his mother to provide transportation (of note, inpatient provider informed pt's mother of the current travel ban due to the snow storm, all fines and risks involved with currently traveling.) Pt provided discharge paperwork and prescriptions. Patient's discharge summary was faxed to the appropriate parties. Patient has been scheduled with the following aftercare appointments:

Centered Primary Care	966 Park St # B6, Stoughton, MA 02072 Phone: (508) 356-7011 Fax: 508-365-2042	
Next Steps: Go on 2/25/2026 Instructions: Patient is scheduled for a hospital follow up appointment with PCP Roberto Amado, MD on 2/25/26 at 11:30 AM.		



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02/18/2026 - Admission (Discharged) in Brockton 3 Behaviorall Health Inpatient Unit (continued)

Notes (continued)

BAMSI Adult Day Treatment Center	Phone: 508-587-5594 Fax: 508-584-4217 www.bamsi.org
Next Steps: Call Instructions: Referral form and records emailed on 2/20/26 for Adult Day Treatment. Recomending completion of a partial hospitalization program (PHP) prior to acceptance. (Please call the day program following completion of a PHP.) Monday - Friday 8:30 a.m. - 4:30 p.m. (Groups held from 9:45 a.m. - 2:45 p.m.) We accept most variations of MassHealth/Medicaid	
Northeast Health Services - Falmouth	340 Gifford Street Falmouth, MA 02540 Phone: 774-375-9993 Fax: (513) 440-8404
Next Steps: Follow up on 2/26/2026 Instructions: Patient is scheduled for a virtual psychiatry appointment with provider Michael Egan, PMHNP on 2/26/26 at 9:20 AM. Note: You've been placed on the wait list for therapy.	
DDS	Address: 60 Main Street, 3rd Floor, Brockton, MA 02301 Phone: (508) 427-5731 Direct: (339) 220-7950
Next Steps: Schedule an appointment as soon as possible for a visit Instructions: Patient's DDS case manager, Christine (339) 220-7950 to reach out post discharge to meet and reconvene services.	
MassHire Greater Brockton Career Center	Address: 34 School Street, Brockton, MA 02301 Phone: (508) 513-3400 Fax: (508) 513-3450 Email: info@masshiregbcc.org Site: https://masshiregbcc.org/
Next Steps: Go to Instructions: Patient to follow up with MassHire for job searching, connection to jobs, training opportunites and resources.	
Gosnold Stoughton PHP	Address: 909 Sumner St Stoughton, MA 02072 Phone: 508-540-6550
Next Steps: Call on 2/25/2026 Instructions: Once PHP re-opens, please call to schedule and intake for a partial hospitalization program.	

Nicole Giardino, LICSW, 2/24/2026.

Electronically signed by Nicole Giardino, LICSW at 2/27/2026 11:31 AM

Progress Note - Nursing/Adult by Dominique Rival, RN at 2/24/2026 0456

Assumed care at 11:30 pm: The patient is alert and oriented x3, was seen pacing in the early hours. He was compliant with HS medications. He spent this shift asleep, no concerns voiced overnight. Continue on 10, 11 and 1s on a 15 min



BMC Yawkey
850 Harrison Ave.
Boston MA02118-4001

Flaherty, Dallas
MRN:3394094
DOB:11/3/1994, Sex: male
SSN:022-78-8206
Visit Date:2/18/2026

02/18/2026 - Admission (Discharged) in Brockton 3 Behaviorall Health Inpatient Unit (continued)

Notes (continued)

safety check. Staff to continue to monitor for changes in mood or behavior and provide support as needed. Patient has a discharge plan for this morning. He slept for about 5 hrs.

Electronically signed by Dominique Rival, RN at 2/24/2026 7:20 AM

Progress Note - Attending by Jared K Trout, MD at 2/23/2026 1329

BMC Brockton Psychiatry Inpatient Progress Note

CHIEF COMPLAINT ("In the patient's own words")

"I feel better."

INTERVAL HISTORY

Accepting medications, no acute behavioral issues. He is out and about in the milieu and pleasant on approach. He appears relatively well-kempt. He says "I feel better." He appears more calm and less labile. He was more calm and cooperative and engaging in the morning and understood that the blizzard conditions make discharge today difficult. He retracted his three day letter and signed another three day letter. E has hopes of being discharged tomorrow.

He signed a three day letter expiring today and signed another three day letter.

PHYSICAL REVIEW OF SYSTEMS

No acute medical issues reported.

MEDICATIONS

Current Facility-Administered Medications

Medication	Dose	Route	Frequency	Provider	Last Rate	Last Admin
• acetaminophen (Ordered as: acetaminophen) tablet 650 mg	650 mg	Oral	Q4H PRN	Juilet Paul, NP		
• amLODIPine (Ordered as: amLODIPine) tablet 10 mg	10 mg	Oral	Daily	Patricia Lopes Varela, NP		10 mg at 02/23/2 6 0832
• benztropine (Ordered as: benztropine) tablet 0.5 mg	0.5 mg	Oral	Q6H PRN	Patricia Lopes Varela, NP		
• busPIRone (Ordered as: busPIRone) tablet 10 mg	10 mg	Oral	BID	Patricia Lopes Varela, NP		10 mg at 02/23/2 6 0833



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02/18/2026 - Admission (Discharged) in Brockton 3 Behaviorall Health Inpatient Unit (continued)

Notes (continued)

• divalproex (Ordered as: divalproex) 24 hr tablet 1,500 mg	1,500 mg	Oral	Daily	Jared K Trout, MD	1,500 mg at 02/23/2 6 0832
• docusate (Ordered as: docusate) capsule 100 mg	100 mg	Oral	BID PRN	Juilet Paul, NP	
• hydroCHLOROthi azide (Ordered as: hydroCHLOROthi azide) tablet 25 mg	25 mg	Oral	Daily	Patricia Lopes Varela, NP	25 mg at 02/23/2 6 0833
• ibuprofen (Ordered as: ibuprofen) tablet 600 mg	600 mg	Oral	QID PRN	Juilet Paul, NP	
• loratadine (Ordered as: loratadine) tablet 10 mg	10 mg	Oral	Daily	Juilet Paul, NP	10 mg at 02/23/2 6 0832
• LORazepam (Ordered as: LORazepam) tablet 1 mg	1 mg	Oral	Q4H PRN	Jared K Trout, MD	1 mg at 02/20/2 6 2057
• melatonin tablet 3 mg	3 mg	Oral	PRN	Patricia Lopes Varela, NP	
• melatonin tablet 9 mg	9 mg	Oral	At bedtime	Patricia Lopes Varela, NP	9 mg at 02/22/2 6 2022
• nicotine polacrilex gum 2 mg	2 mg	Oral	Q2H PRN	Patricia Lopes Varela, NP	
• OLANZapine (Ordered as: OLANZapine) tablet 10 mg	10 mg	Oral	BID	Jared K Trout, MD	10 mg at 02/23/2 6 0833
• OLANZapine (Ordered as: OLANZapine) tablet 5 mg	5 mg	Oral	BID PRN	Jared K Trout, MD	
• pantoprazole (Ordered as: pantoprazole) EC tablet 40 mg	40 mg	Oral	Daily	Patricia Lopes Varela, NP	40 mg at 02/22/2 6 0609
• sodium chloride	1	Nasal	Q4H PRN	Juilet Paul, NP	



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02/18/2026 - Admission (Discharged) in Brockton 3 Behaviorall Health Inpatient Unit (continued)

Notes (continued)

0.65 % nasal spray
spray 1 spray

ALLERGIES

No Known Allergies

VITALS

Vitals:

02/23/26 0832
BP: 138/89
Pulse:
Resp:
Temp:
SpO2:

LABS

Vitals:

02/23/26 0832
BP: 138/89
Pulse:
Resp:
Temp:
SpO2:

MENTAL STATUS EXAMINATION

Patient is awake, alert, and oriented x 3. Appearance is disheveled. He denies suicidal ideation. He denies homicidal ideation. He denies auditory hallucinations. He denies visual hallucinations. Mood is irritable and labile. Affect is congruent, irritable, labile, intense. Thought process is disorganized and circumstantial. Thought content shows marked paranoid ideation. Speech is rapid and pressured and loud. He appears psychomotor agitated, yelling and pacing the unit and pointing at staff members. Insight is poor. Judgment is poor.

ASSESSMENT/TREATMENT

Mr. Flaherty is a 31 year-old man BIBA on a sec 12 after his mother called the police reporting that pt kicked in her door, was yelling at her and threatening to kill her. Pt's mother states he woke up "angry" today. Pt denies HI/AH/VH, reports passive SI, feeling so overwhelmed "I can't take it anymore".

He presents after a high-risk domestic incident involving threats to kill his mother and a reported knife threat. Even though he minimizes intent and describes it as accidental and sheathed, the reported behavior represents significant acute risk given escalation, impulsivity, agitation, and access to a weapon during conflict. His presentation is characterized by marked irritability, hyperv verbal speech, severe anxiety, poor sleep, appetite reduction, and impaired behavioral control. He denies current psychotic symptoms and denies current suicidal ideation. He has a history of a suicide attempt, chronic psychosocial stressors, and limited supports.



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02/18/2026 - Admission (Discharged) in Brockton 3 Behavioral Health Inpatient Unit (continued)

Notes (continued)

Accepting medications. More calm and cooperative today. Less labile and irritable. More rational - he understands that the blizzard makes discharge today treacherous and he retracted his three day letter expiring today and signed another. Plan is for discharge tomorrow (2/24/26) when the weather clears. We will continue olanzapine to 10 mg at BID. We will continue lorazepam as needed for anxiety and manic symptoms. We will consolidate depakote to 1500 mg daily. We will follow up depakote level tomorrow. We will get collateral as able. We will provide structure and support. We will continue to observe.

He signed a three day letter expiring today and signed another three day letter.

DIAGNOSIS(ES)

Principal Problem:

Bipolar disorder with depression (CMS/HHS/HCC)

Active Problems:

Cannabis use disorder

LEGAL STATUS:

Legal Status Orders

Legal

Section 10 & 11, 3 day letter

FRESH AIR:

Eligible for Fresh Air

I certify that the inpatient psychiatric facility services furnished since the previous certification or recertification were, and continue to be, medically necessary for irritability, lability, erratic behavior, and manic symptoms. Hospital records indicate that the services furnished were either: intensive treatment services, admission and related services necessary for diagnostic study, or equivalent services. I certify that the patient continues to need, on a daily basis, active treatment furnished directly by or requiring the supervision of inpatient psychiatric facility personnel. I estimate 10 days of hospitalization is necessary for proper treatment of the patient. My plans for post-hospital care for this patient are TBD.

Jared K Trout, MD 2/23/2026 1:29 PM

Electronically signed by Jared K Trout, MD at 2/23/2026 1:35 PM

Progress Note - Nursing/Adult by Albert Smith III, RN at 2/23/2026 1059

Diagnosis:

Bipolar disorder with depression

Cannabis use disorder

Maintained Q 15 Min Checks

Printed on 4/10/26 10:45 AM

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02/18/2026 - Admission (Discharged) in Brockton 3 Behaviorall Health Inpatient Unit (continued)

Notes (continued)

Legal: 10/11CV

Days:

Denies SI/HI/SIB/AH/VH

In room - out for needs
Calm and cooperative on approach
Minimal engagement
Looking forward to discharge

Community Meeting:

Groups:

Fresh Air:

Medications:

A/O

PRN:

Did not utilize

New Orders:

N.N.O.

Nursing Communication:

Cardiac Diet

VS parameters - notify provider

Vitals:

02/23/26 0832

BP: 138/89

Pulse:

Resp:

Temp:

SpO2:

Discharge Plans:

3 Day Notice

Electronically signed by Albert Smith III, RN at 2/23/2026 6:32 PM

Progress Note - Attending by Chuck Ogbodo, NP at 2/22/2026 1211

BMC Brockton Psychiatry Inpatient Progress Note

CHIEF COMPLAINT ("In the patient's own words")

"Feel a lot better than when I came in; feel like I'm ready to go home."

INTERVAL HISTORY

2/22/26- provider met with pt in his room and easily awoken. He reports coming out for breakfast. Pt endorsed feeling anxious because he is unsure if he will be discharged on his 3days that is up tomorrow, he consistently asked provider

Printed on 4/10/26 10:45 AM

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02/18/2026 - Admission (Discharged) in Brockton 3 Behaviorall Health Inpatient Unit (continued)

Notes (continued)

to decide if he will be discharged tomorrow due to snow and not willing to wait till tomorrow to discuss with treatment team.

2/21/26-Accepting medications, no acute behavioral issues. patient reports significant improvement compared to prior presentation and expresses readiness for discharge. He is calm and cooperative and engaging during session. He stated his mother wants to talk about him coming home with primary team. He denies SI/HI and AVH.

He signed a three day letter expiring on 2/23/26.

PHYSICAL REVIEW OF SYSTEMS

No acute medical issues reported.

MEDICATIONS

Current Facility-Administered Medications

Medication	Dose	Route	Frequency	Provider	Last Rate	Last Admin
• acetaminophen (Ordered as: acetaminophen) tablet 650 mg	650 mg	Oral	Q4H PRN	Juilet Paul, NP		
• amLODIPine (Ordered as: amLODIPine) tablet 10 mg	10 mg	Oral	Daily	Patricia Lopes Varela, NP		10 mg at 02/22/2 6 0924
• benztropine (Ordered as: benztropine) tablet 0.5 mg	0.5 mg	Oral	Q6H PRN	Patricia Lopes Varela, NP		
• busPIRone (Ordered as: busPIRone) tablet 10 mg	10 mg	Oral	BID	Patricia Lopes Varela, NP		10 mg at 02/22/2 6 0925
• divalproex (Ordered as: divalproex) 24 hr tablet 1,500 mg	1,500 mg	Oral	Daily	Jared K Trout, MD		1,500 mg at 02/22/2 6 0925
• docusate (Ordered as: docusate) capsule 100 mg	100 mg	Oral	BID PRN	Juilet Paul, NP		
• hydroCHLOROthi azide (Ordered as:	25 mg	Oral	Daily	Patricia Lopes Varela, NP		25 mg at 02/22/2



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02/18/2026 - Admission (Discharged) in Brockton 3 Behaviorall Health Inpatient Unit (continued)

Notes (continued)

hydroCHLOROthi azide) tablet 25 mg						6 0925
• ibuprofen (Ordered as: ibuprofen) tablet 600 mg	600 mg	Oral	QID PRN	Juilet Paul, NP		
• loratadine (Ordered as: loratadine) tablet 10 mg	10 mg	Oral	Daily	Juilet Paul, NP	10 mg at 02/22/2 6 0925	
• LORazepam (Ordered as: LORazepam) tablet 1 mg	1 mg	Oral	Q4H PRN	Jared K Trout, MD	1 mg at 02/20/2 6 2057	
• melatonin tablet 3 mg	3 mg	Oral	PRN	Patricia Lopes Varela, NP		
• melatonin tablet 9 mg	9 mg	Oral	At bedtime	Patricia Lopes Varela, NP	9 mg at 02/21/2 6 2102	
• nicotine polacrilex gum 2 mg	2 mg	Oral	Q2H PRN	Patricia Lopes Varela, NP		
• OLANZapine (Ordered as: OLANZapine) tablet 10 mg	10 mg	Oral	BID	Jared K Trout, MD	10 mg at 02/22/2 6 0924	
• OLANZapine (Ordered as: OLANZapine) tablet 5 mg	5 mg	Oral	BID PRN	Jared K Trout, MD		
• pantoprazole (Ordered as: pantoprazole) EC tablet 40 mg	40 mg	Oral	Daily	Patricia Lopes Varela, NP	40 mg at 02/22/2 6 0609	
• sodium chloride 0.65 % nasal spray 1 spray	1 spray	Nasal	Q4H PRN	Juilet Paul, NP		
• tuberculin injection 5 Units	5 Units	Intradermal	Once	Patricia Lopes Varela, NP		

ALLERGIES

No Known Allergies

VITALS

Vitals:



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02/18/2026 - Admission (Discharged) in Brockton 3 Behaviorall Health Inpatient Unit (continued)

Notes (continued)

02/22/26 0928
BP: 123/91
Pulse: (!) 127
Resp: 18
Temp: 98 °F (36.7 °C)
SpO2: 98%

LABS

Vitals:

02/22/26 0928
BP: 123/91
Pulse: (!) 127
Resp: 18
Temp: 98 °F (36.7 °C)
SpO2: 98%

MENTAL STATUS EXAMINATION

Patient is awake, alert, and oriented x 3. Appearance is disheveled. He denies suicidal ideation. He denies homicidal ideation. He denies auditory hallucinations. He denies visual hallucinations. Mood is irritable. Affect is congruent, irritable, intense. Thought process is perseverating, and circumstantial. Thought content shows marked paranoid ideation. Speech is rapid and loud. Insight is poor. Judgment is poor.

ASSESSMENT/TREATMENT

Mr. Flaherty is a 31 year-old man BIBA on a sec 12 after his mother called the police reporting that pt kicked in her door, was yelling at her and threatening to kill her. Pt's mother states he woke up "angry" today. Pt denies HI/AH/VH, reports passive SI, feeling so overwhelmed "I can't take it anymore".

He presents after a high-risk domestic incident involving threats to kill his mother and a reported knife threat. Even though he minimizes intent and describes it as accidental and sheathed, the reported behavior represents significant acute risk given escalation, impulsivity, agitation, and access to a weapon during conflict.

2/22/26- pt is anxious, mildly irritable and lacks patience, remains impulsive and appears to minimize symptoms, he reports incident with his mother was a mistake nit intentional. there is some increased intensity noted when asking about possible discharge, he is preoccupied and perseverating on discharge . He denies AVH/SI/HI.

2/21/26-Today pt calm, cooperative good behavioral control. Patient reports mood is good and sleep is good. Denies auditory hallucinations today and denies visual hallucinations. Denies suicidal ideation and denies homicidal ideation.

He has a history of a suicide attempt, chronic psychosocial stressors, and limited supports.

Three day letter expires on 2/23/26.

DIAGNOSIS(ES)



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02/18/2026 - Admission (Discharged) in Brockton 3 Behavioral Health Inpatient Unit (continued)

Notes (continued)

Principal Problem:

Bipolar disorder with depression (CMS/HHS/HCC)

Active Problems:

Cannabis use disorder

LEGAL STATUS:

Legal Status Orders

Legal

Section 10 & 11, 3 day letter

FRESH AIR:

Eligible for Fresh Air

I certify that the inpatient psychiatric facility services furnished since the previous certification or recertification were, and continue to be, medically necessary for irritability, lability, erratic behavior, and manic symptoms. Hospital records indicate that the services furnished were either: intensive treatment services, admission and related services necessary for diagnostic study, or equivalent services. I certify that the patient continues to need, on a daily basis, active treatment furnished directly by or requiring the supervision of inpatient psychiatric facility personnel. I estimate 10 days of hospitalization is necessary for proper treatment of the patient. My plans for post-hospital care for this patient are TBD.

Chuck Ogbodo, NP 2/22/2026 12:11 PM

Electronically signed by Chuck Ogbodo, NP at 2/22/2026 6:40 PM

Progress Note - Nursing/Adult by Vincent Sweeney, RN at 2/22/2026 1153

Patient is alert and oriented. Taking prescribed meds. Reports mood as being anxious. Has 3 day notice that expires tomorrow and expresses concern that impending snow storm will interfere with discharge. States that events leading to admission were a mistake and will not happen again. Will continue to monitor for safety.

Electronically signed by Vincent Sweeney, RN at 2/22/2026 11:55 AM

Progress Note - Nursing/Adult by Sandra Alves, RN at 2/20/2026 1413

Legal Status: 10/11 3 day notice exp Mon 2/23/26

Safety Checks:

Subjective:

Patient reports no current thoughts of self-harm or harm to others. Denies auditory or visual hallucinations. Denies



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02/18/2026 - Admission (Discharged) in Brockton 3 Behaviorall Health Inpatient Unit (continued)

Notes (continued)

delusions or paranoia. Patient expresses feeling "I just want to go home" and reports sleeping and eating adequately. No new complaints voiced.

Objective:

Patient observed to have irritable edge. Affect appropriate to content, mood-angry. Speech clear and coherent. Thought process linear and goal-directed. No overt signs of internal preoccupation or responding to internal stimuli. Patient engaging appropriately with peers and staff. Eating meals and drinking fluids without difficulty. Has not been participating in groups/programming. No behavioral concerns noted. Medications taken as prescribed without resistance. Pt had VPA labs done today, results pending.

Assessment:

No evidence of SI/HI/AH/VH. Compliant with treatment plan and medications. Meeting basic needs. No acute distress observed.

Plan:

Plan of care will be ongoing, no acute concerns at this time. Encourage continued engagement in therapeutic activities. Support discharge planning as appropriate.

Electronically signed by Sandra Alves, RN at 2/20/2026 2:18 PM

Progress Note - Attending by Jared K Trout, MD at 2/20/2026 1152

BMC Brockton Psychiatry Inpatient Progress Note

CHIEF COMPLAINT ("In the patient's own words")

"I don't belong in a place like this."

INTERVAL HISTORY

Accepting medications, no acute behavioral issues. He was more calm and cooperative and engaging in the morning and was saying he could leave on "Monday or Tuesday." Mother does appear to be willing to have him back in the home. He also consented to outpatient appointments. He denies SI/HI and AVH.

Later in the afternoon he became more intense and tearful, saying "I don't belong here" and asking to be discharged immediately. He was agitated but less so than yesterday. He agreed to take olanzapine 10 mg PO and lorazepam 2 mg PO.

He signed a three day letter expiring on 2/23/26.

PHYSICAL REVIEW OF SYSTEMS

No acute medical issues reported.

MEDICATIONS

Current Facility-Administered Medications

Medication	Dose	Route	Frequency	Provider	Last Rate	Last Admin
• acetaminophen	650	Oral	Q4H PRN	Juilet Paul, NP		

Printed on 4/10/26 10:45 AM

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02/18/2026 - Admission (Discharged) in Brockton 3 Behaviorall Health Inpatient Unit (continued)

Notes (continued)

(Ordered as: acetaminophen) tablet 650 mg	mg					
• amLODIPine (Ordered as: amLODIPine) tablet 10 mg	10 mg	Oral	Daily	Patricia Lopes Varela, NP	10 mg at 02/20/2 6 1033	
• benztropine (Ordered as: benztropine) tablet 0.5 mg	0.5 mg	Oral	Q6H PRN	Patricia Lopes Varela, NP		
• busPIRone (Ordered as: busPIRone) tablet 10 mg	10 mg	Oral	BID	Patricia Lopes Varela, NP	10 mg at 02/20/2 6 1033	
• divalproex (Ordered as: divalproex) 24 hr tablet 1,500 mg	1,500 mg	Oral	Daily	Jared K Trout, MD	1,500 mg at 02/20/2 6 1033	
• docusate (Ordered as: docusate) capsule 100 mg	100 mg	Oral	BID PRN	Juilet Paul, NP		
• hydroCHLOROthi azide (Ordered as: hydroCHLOROthi azide) tablet 25 mg	25 mg	Oral	Daily	Patricia Lopes Varela, NP	25 mg at 02/20/2 6 1033	
• ibuprofen (Ordered as: ibuprofen) tablet 600 mg	600 mg	Oral	QID PRN	Juilet Paul, NP		
• loratadine (Ordered as: loratadine) tablet 10 mg	10 mg	Oral	Daily	Juilet Paul, NP	10 mg at 02/20/2 6 1033	
• LORazepam (Ordered as: LORazepam) tablet 2 mg	2 mg	Oral	Q4H PRN	Jared K Trout, MD		
• melatonin tablet 3 mg	3 mg	Oral	PRN	Patricia Lopes Varela, NP		
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02/18/2026 - Admission (Discharged) in Brockton 3 Behaviorall Health Inpatient Unit (continued)

Notes (continued)

• nicotine polacrilex gum 2 mg	2 mg	Oral	Q2H PRN	Patricia Lopes Varela, NP	
• OLANZapine (Ordered as: OLANZapine) tablet 10 mg	10 mg	Oral	At bedtime	Jared K Trout, MD	10 mg at 02/19/26 2041
• OLANZapine (Ordered as: OLANZapine) tablet 5 mg	5 mg	Oral	Q8H PRN	Jared K Trout, MD	
• pantoprazole (Ordered as: pantoprazole) EC tablet 40 mg	40 mg	Oral	Daily	Patricia Lopes Varela, NP	40 mg at 02/20/26 0613
• sodium chloride 0.65 % nasal spray 1 spray	1	Nasal spray	Q4H PRN	Juilet Paul, NP	
• tuberculin injection 5 Units	5 Units	Intradermal	Once	Patricia Lopes Varela, NP	

ALLERGIES

No Known Allergies

VITALS

Vitals:

02/20/26 1035
BP: 175/94
Pulse: 86
Resp: 18
Temp: 98.3 °F (36.8 °C)
SpO2: 97%

LABS

Vitals:

02/20/26 1035
BP: 175/94
Pulse: 86
Resp: 18
Temp: 98.3 °F (36.8 °C)
SpO2: 97%

MENTAL STATUS EXAMINATION



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02/18/2026 - Admission (Discharged) in Brockton 3 Behaviorall Health Inpatient Unit (continued)

Notes (continued)

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ASSESSMENT/TREATMENT

Mr. Flaherty is a 31 year-old man BIBA on a sec 12 after his mother called the police reporting that pt kicked in her door, was yelling at her and threatening to kill her. Pt's mother states he woke up "angry" today. Pt denies HI/AH/VH, reports passive SI, feeling so overwhelmed "I can't take it anymore".

He presents after a high-risk domestic incident involving threats to kill his mother and a reported knife threat. Even though he minimizes intent and describes it as accidental and sheathed, the reported behavior represents significant acute risk given escalation, impulsivity, agitation, and access to a weapon during conflict. His presentation is characterized by marked irritability, hypervocal speech, severe anxiety, poor sleep, appetite reduction, and impaired behavioral control. He denies current psychotic symptoms and denies current suicidal ideation. He has a history of a suicide attempt, chronic psychosocial stressors, and limited supports.

Accepting medications. More calm and cooperative in the morning. More agitated in the afternoon. We will increase olanzapine to 10 mg at BID. We will continue lorazepam as needed for anxiety and manic symptoms. We will consolidate depakote to 1500 mg daily. We will follow up depakote level tomorrow. We will get collateral as able. We will provide structure and support. We will continue to observe.

Three day letter expires on 2/23/26.

DIAGNOSIS(ES)

Principal Problem:

Bipolar disorder with depression (CMS/HHS/HCC)

Active Problems:

Cannabis use disorder

LEGAL STATUS:

Legal Status Orders

Legal

Section 10 & 11, 3 day letter

FRESH AIR:

Eligible for Fresh Air

I certify that the inpatient psychiatric facility services furnished since the previous certification or recertification were, and continue to be, medically necessary for irritability, lability, erratic behavior, and manic symptoms. Hospital records indicate that the services furnished were either: intensive treatment services, admission and related services necessary for diagnostic study, or equivalent services. I certify that the patient continues to need, on a daily basis, active treatment furnished directly by or requiring the supervision of inpatient psychiatric facility personnel.



BMC Yawkey
850 Harrison Ave.
Boston MA02118-4001

Flaherty, Dallas
MRN:3394094
DOB:11/3/1994,Sex:male
SSN:022-78-8206
Visit Date:2/18/2026

02/18/2026 - Admission (Discharged) in Brockton 3 Behaviorall Health Inpatient Unit (continued)

Notes (continued)

I estimate 10 days of hospitalization is necessary for proper treatment of the patient. My plans for post-hospital care for this patient are TBD.

Jared K Trout, MD 2/20/2026 11:52 AM

Electronically signed by Jared K Trout, MD at 2/20/2026 4:14 PM

Progress Note - Nursing/Adult by Rebecca Njonjo, RN at 2/20/2026 0555

Patient is a 31-year-old male, admitted yesterday on Section 10/11, During the 7p-7a shift, patient was alert and oriented, somewhat calm and cooperative with care. Self-presented for HS medications and was compliant with all scheduled medications. Utilized PRN medication as needed. Earlier in the day shift, patient reportedly became verbally aggressive and demanded discharge; however, no behavioral escalation was observed during this shift. Patient remained mostly isolated in his room throughout the evening and maintained behavioral control. Denies SI/HI/AH/VH. Slept approximately 8 hours overnight without disturbance. Remains on safety checks per protocol. Staff will continue to monitor mood, behavior, and provide support as needed.

Electronically signed by Rebecca Njonjo, RN at 2/20/2026 5:55 AM

H&P by Juliet Paul, NP at 2/19/2026 1152

Brockton Psych Medicine H&P

MEDICINE HISTORY & PHYSICAL

Admit Date: Feb 18, 2026

Attending: Jared K Trout, MD

PCP Provider/Site: Akash D Patel, MD / 401 N BEDFORD ST Suite 210 / EAST BRIDGEWATER MA 02333

Chief Complaint: Pt states "My mother"

HPI: Pt presented to BMC Brockton Psych Hospital on 2/18/2026 from BMC South.

He presented to BMC South 2/17/2026 from home after mother called 911 due to pt threatening her with knife, kicking and breaking door. Patient states that he got frustrated because he was stuck in his house without any work to go to. Reported he has been attempting to find a job, but has been having difficulty doing that which is furthers his frustration.

Pt and his mother report pt was born with the cord wrapped around his neck 4 times, 0 APGAR and required brain surgery/a shunt. Pt's mother reports this led to some developmental/cognitive delays and pt has also struggled with mental health issues throughout his life. Pt medically cleared.

Living with mother

Urine tox + Cannabis

Denies active HI/SI/SIB/AH/VH



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02/18/2026 - Admission (Discharged) in Brockton 3 Behaviorall Health Inpatient Unit (continued)

Notes (continued)

Medications Prior to Admission

Medication	Sig	Dispense	Refill	Last Dose/Taking
• amLODIPine (NORVASC) 10 mg tablet	Take 10 mg by mouth daily.			
• busPIRone (BUSPAR) 10 mg tablet	Take 10 mg by mouth 2 (two) times a day.			
• divalproex 500 MG 24 hr tablet	Take 500 mg by mouth 3 (three) times a day.			
• hydroCHLORothiazide (HYDRODIURIL) 25 mg tablet	Take 25 mg by mouth daily.			
• MELATONIN ORAL	Take 3 mg by mouth as needed (insomnia).			
• OLANZapine (ZYPREXA) 5 MG tablet	Take 5 mg by mouth nightly.			
• omeprazole (PRILOSEC) 20 mg capsule	Take 20 mg by mouth daily.			

Vitals:

02/19/26 0836
BP: 127/85
Pulse: 79
Resp: 16
Temp: 97.2 °F (36.2 °C)
SpO2: 97%

Patient Active Problem List

Diagnosis	Date Noted
• Bipolar disorder with depression (CMS/HHS/HCC)	02/18/2026
• Cannabis use disorder	02/18/2026

Past Medical History:

Diagnosis	Date
• Ankle sprain	
• Anxiety	
• Bipolar affective disorder (CMS/HHS/HCC)	
• Cannabis use disorder	
• Depression	
• Developmental delay	
• <i>r/t to being born with cord around neck that required shunt placement per mother</i>	
• GERD (gastroesophageal reflux disease)	
• HTN (hypertension)	



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02/18/2026 - Admission (Discharged) in Brockton 3 Behaviorall Health Inpatient Unit (continued)

Notes (continued)

- Insomnia
- Nasal congestion
- Seasonal allergies
- Tobacco use disorder

Past Surgical History:

Procedure	Laterality	Date
• cerberal shunt placed as child <i>mother report pt was born with the cord wrapped around his neck 4 times, 0 APGAR and required brain surgery/a shunt.</i>		
• cerberal shunt revision 2007 per pt		

No family history on file.

Current Facility-Administered Medications

Medication	Dose	Route	Frequency	Provider	Last Rate	Last Admin
• acetaminophen tablet 650 mg	650 mg	Oral	Q4H PRN	Juilet Paul, NP		
• amLODIPine (Ordered as: amLODIPine) tablet 10 mg	10 mg	Oral	Daily	Patricia Lopes Varela, NP		10 mg at 02/19/2 6 0905
• benztropine (Ordered as: benztropine) tablet 0.5 mg	0.5 mg	Oral	Q6H PRN	Patricia Lopes Varela, NP		
• busPIRone (Ordered as: busPIRone) tablet 10 mg	10 mg	Oral	BID	Patricia Lopes Varela, NP		10 mg at 02/19/2 6 0905
• divalproex (Ordered as: divalproex) 24 hr tablet 500 mg	500 mg	Oral	TID	Patricia Lopes Varela, NP		500 mg at 02/19/2 6 0905
• docusate capsule 100 mg	100 mg	Oral	BID PRN	Juilet Paul, NP		
• hydroCHLOROthi azide (Ordered as: hydroCHLOROthi azide) tablet 25 mg	25 mg	Oral	Daily	Patricia Lopes Varela, NP		25 mg at 02/19/2 6 0905
• ibuprofen tablet 600 mg	600 mg	Oral	QID PRN	Juilet Paul, NP		
• loratadine	10 mg	Oral	Daily	Juilet Paul, NP		



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02/18/2026 - Admission (Discharged) in Brockton 3 Behaviorall Health Inpatient Unit (continued)

Notes (continued)

(Ordered as: loratadine) tablet 10 mg						
• melatonin tablet 3 mg	3 mg	Oral	PRN	Patricia Lopes Varela, NP		
• melatonin tablet 9 mg	9 mg	Oral	At bedtime	Patricia Lopes Varela, NP		
• nicotine polacrilex gum 2 mg	2 mg	Oral	Q2H PRN	Patricia Lopes Varela, NP		
• OLANZapine (Ordered as: OLANZapine) disintegrating tablet 5 mg	5 mg	Oral	Q6H PRN	Patricia Lopes Varela, NP	5 mg at 02/19/2 6 0907	
• OLANZapine (Ordered as: OLANZapine) tablet 5 mg	5 mg	Oral	At bedtime	Patricia Lopes Varela, NP		
• pantoprazole (Ordered as: pantoprazole) EC tablet 40 mg	40 mg	Oral	Daily	Patricia Lopes Varela, NP	40 mg at 02/19/2 6 0655	
• sodium chloride 0.65 % nasal spray 1 spray	1 spray	Nasal	Q4H PRN	Juilet Paul, NP		
• tuberculin injection 5 Units	5 Units	Intradermal	Once	Patricia Lopes Varela, NP		

Allergies as of 02/18/2026

- (Not on File)

Review of Systems

Constitutional: Negative.

HENT: Negative.

Eyes: Negative.

Respiratory: Negative.

Cardiovascular: Negative.

Gastrointestinal: Negative.

Endocrine: Negative.

Genitourinary: Negative.

Musculoskeletal: Negative.

Skin: Negative.

Allergic/Immunologic: Negative.

Neurological: Negative.

Hematological: Negative.



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02/18/2026 - Admission (Discharged) in Brockton 3 Behavioral Health Inpatient Unit (continued)

Notes (continued)

Psychiatric/Behavioral: Positive for sleep disturbance. Negative for agitation, behavioral problems, confusion, decreased concentration, dysphoric mood, hallucinations, self-injury and suicidal ideas. The patient is not nervous/anxious and is not hyperactive.

Physical Exam HENT:

Ears:

Comments: **Gross vision intact**

Nose: Congestion present.

Comments: **Some nasal congestion**

Eyes:

Conjunctiva/sclera: Conjunctivae normal.

Pupils: Pupils are equal, round, and reactive to light.

Comments: **Vision intact**

L eye higher compared to R eyes

Cardiovascular:

Rate and Rhythm: Normal rate and regular rhythm.

Pulses: Normal pulses.

Heart sounds: Normal heart sounds.

Pulmonary:

Effort: Pulmonary effort is normal. No respiratory distress.

Breath sounds: Normal breath sounds. No stridor. No wheezing or rhonchi.

Abdominal:

General: Bowel sounds are normal. There is no distension.

Palpations: Abdomen is soft. There is no mass.

Tenderness: There is no abdominal tenderness. There is no guarding or rebound.

Hernia: No hernia is present.

Comments: **Abd supple**

Genitourinary:

Comments: **Deferred**

Musculoskeletal:

General: Normal range of motion.

Cervical back: Normal range of motion.

Neurological:

Mental Status: He is alert and oriented to person, place, and time. Mental status is at baseline.

Comments: **CN 2-12**

Visual Field normal in all 4 quadrants, pupil round and reactive to light, extraocular movements intact bilaterally

Facial muscles normal and equal bil, facial sensation intact

Tongue midline

Hearing intact presents normal and intact bil

Voice normal

Shoulder shrug strong and equal

Biceps, brachioradialis, triceps, patellar, achilles 2+/4 Bil

No clonus.



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02/18/2026 - Admission (Discharged) in Brockton 3 Behaviorall Health Inpatient Unit (continued)

Notes (continued)

Plantar reflex downward bilaterally

Sensation intact bil

Gait steady with normal base

Ambulating and R tippy toe at times

Skin:

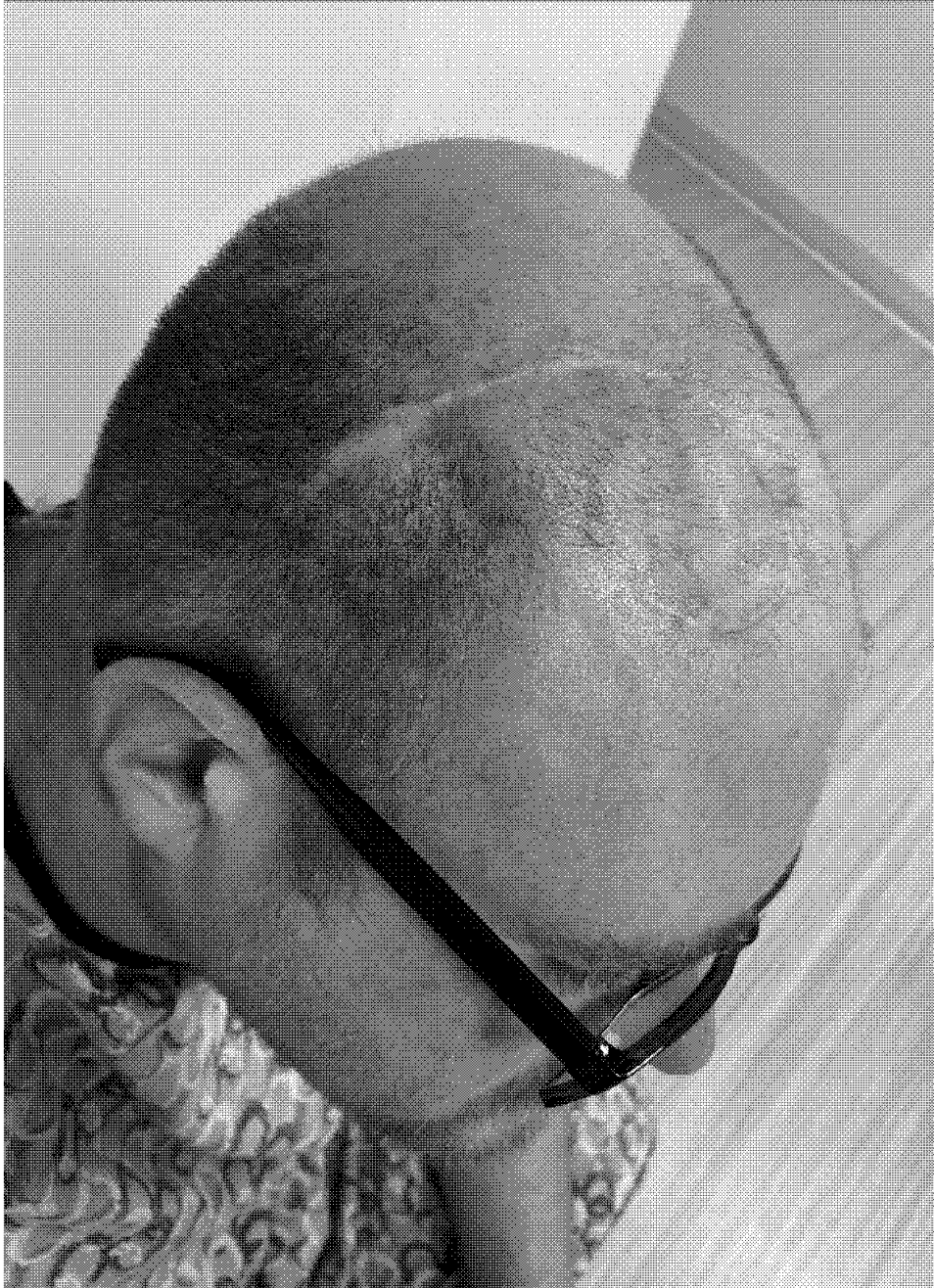
General: Skin is warm.

Psychiatric:

Comments: **Anxious**

02/18/2026 - Admission (Discharged) in Brockton 3 Behaviorall Health Inpatient Unit (continued)

Notes (continued)





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02/18/2026 - Admission (Discharged) in Brockton 3 Behaviorall Health Inpatient Unit (continued)

Notes (continued)

Smoke Cessation:

Does Patient want a referral to Quit Works?: No
Does Patient want smoking cessation medications prescribed on discharge?: No
Ready to Quit?: No
Counseling offered: Yes
Medications offered : Yes
Medications offered: Nicotine Patch
Smoking cessation medication prescribed: Yes

Assessment/Plan:

Bipolar

Anxiety

Insomnia

Depression

Developmental delay

Cont Buspirone 10mg BID
Cont divalproex 24hrs 500mg TID
Cont melatonin 9mg at HS
Cont olanzapine 5mg at HS
Cont benztropine 0.5mg Q 6hrs prn extrapyramidal symptoms
Cont melatonin 3mg nightly prn insomnia
Cont olanzapine 5mg Q 6hrs prn agitation
Managed by psychic team
Labs per psychic and protocol
Encourage group
CTM mood, behavior,sleep wake pattern

HTN

Cont amlodipine 10mg daily
Cont hydrochlorothiazide 25mg daily
CTM b/p, hr, cardiac status

Tobacco use

Cont nicotine gum 2mg Q 2hrs prn smoking cessation
Encourage cessation
CTM respiratory status and complications

GERD

Cont pantoprazole 40mg daily
CTM Gi status

Seasonal allergies

Nasal congestion

Add Claritin 10mg daily
Add sodium 0.65% 1 spray each nostril Q 4hrs prn nasal congestion
CTM for improvement



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02/18/2026 - Admission (Discharged) in Brockton 3 Behavioral Health Inpatient Unit (continued)

Notes (continued)

Cannabis use

No FDA approved detox
Encourage cessation
CTM for craving and complications

Cerebral shut

Pt and his mother report pt was born with the cord wrapped around his neck 4 times, 0 APGAR and required brain surgery/a shunt.

Place as child per pt and followed at New England Medical center

CTM for complication and mental status

House meds

Cont acetaminophen 650mg Q 4hrs prn pain/fever/HA

Cont colace 100mg BID prn constipation

Cont ibuprofen 600mg QID prn pain/fever/HA

Juilet Paul, NP

Electronically signed by Juilet Paul, NP at 2/19/2026 12:26 PM

Progress Note - Attending by Jared K Trout, MD at 2/19/2026 0933

BMC Brockton Psychiatry Inpatient Progress Note

CHIEF COMPLAINT ("In the patient's own words")

"Driving myself nuts."

INTERVAL HISTORY

Accepting medications. Irritable, agitated, labile, verbally abusive, demanding to leave the hospital immediately.

He says that he is "constantly stuck in the house" because "no one's hiring" and so he is "in the house constantly" and is "driving myself nuts."

He admits he made a mistake in threatening his mother and denies HI but he appears quite angry with her. He says "She doesn't want me to be happy" and that "She uses it (perhaps actions or mental illness) against me as a weapon."

He gets more animated and says "She says she the victim? F--- you, b---! I'm a victim!"

At the same time, he insists on being discharged immediately so that he can "Go home an relax" and says "I've learned my lesson."

When he learns that he is not being discharged today, he becomes angry, vulgar, verbally abusive, and paces the unit, insisting the door be unlocked and threatening legal action. He was ultimately able to be redirected with effort and accepted olanzapine 10 mg PO and lorazepam 2 mg PO.



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02/18/2026 - Admission (Discharged) in Brockton 3 Behaviorall Health Inpatient Unit (continued)

Notes (continued)

He says that he was hospitalized at Westboro in 7/2025.

He apparently has DDS services.

He signed a three day letter expiring on 2/23/26.

PHYSICAL REVIEW OF SYSTEMS

No acute medical issues reported.

MEDICATIONS

Current Facility-Administered Medications

Medication	Dose	Route	Frequency	Provider	Last Rate	Last Admin
• acetaminophen (Ordered as: acetaminophen) tablet 650 mg	650 mg	Oral	Q6H PRN	Patricia Lopes Varela, NP		
• amLODIPine (Ordered as: amLODIPine) tablet 10 mg	10 mg	Oral	Daily	Patricia Lopes Varela, NP		10 mg at 02/19/2 6 0905
• benztropine (Ordered as: benztropine) tablet 0.5 mg	0.5 mg	Oral	Q6H PRN	Patricia Lopes Varela, NP		
• busPIRone (Ordered as: busPIRone) tablet 10 mg	10 mg	Oral	BID	Patricia Lopes Varela, NP		10 mg at 02/19/2 6 0905
• divalproex (Ordered as: divalproex) 24 hr tablet 500 mg	500 mg	Oral	TID	Patricia Lopes Varela, NP		500 mg at 02/19/2 6 0905
• docusate (Ordered as: docusate) capsule 100 mg	100 mg	Oral	Nightly PRN	Patricia Lopes Varela, NP		
• hydroCHLOROthi azide (Ordered as: hydroCHLOROthi azide) tablet 25 mg	25 mg	Oral	Daily	Patricia Lopes Varela, NP		25 mg at 02/19/2 6 0905



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02/18/2026 - Admission (Discharged) in Brockton 3 Behaviorall Health Inpatient Unit (continued)

Notes (continued)

• melatonin tablet 3 mg	3 mg	Oral	PRN	Patricia Lopes Varela, NP	
• melatonin tablet 9 mg	9 mg	Oral	At bedtime	Patricia Lopes Varela, NP	
• nicotine polacrilex gum 2 mg	2 mg	Oral	Q2H PRN	Patricia Lopes Varela, NP	
• OLANZapine (Ordered as: OLANZapine) disintegrating tablet 5 mg	5 mg	Oral	Q6H PRN	Patricia Lopes Varela, NP	5 mg at 02/19/2 6 0907
• OLANZapine (Ordered as: OLANZapine) tablet 5 mg	5 mg	Oral	At bedtime	Patricia Lopes Varela, NP	
• pantoprazole (Ordered as: pantoprazole) EC tablet 40 mg	40 mg	Oral	Daily	Patricia Lopes Varela, NP	40 mg at 02/19/2 6 0655
• tuberculin injection 5 Units	5 Units	Intradermal	Once	Patricia Lopes Varela, NP	

ALLERGIES

No Known Allergies

VITALS

Vitals:

02/19/26 0836
BP: 127/85
Pulse: 79
Resp: 16
Temp: 97.2 °F (36.2 °C)
SpO2: 97%

LABS

Vitals:

02/19/26 0836
BP: 127/85
Pulse: 79
Resp: 16
Temp: 97.2 °F (36.2 °C)
SpO2: 97%



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02/18/2026 - Admission (Discharged) in Brockton 3 Behaviorall Health Inpatient Unit (continued)

Notes (continued)

MENTAL STATUS EXAMINATION

Patient is awake, alert, and oriented x 3. Appearance is disheveled. He denies suicidal ideation. He denies homicidal ideation. He denies auditory hallucinations. He denies visual hallucinations. Mood is irritable and labile. Affect is congruent, irritable, labile, intense. Thought process is disorganized and circumstantial. Thought content shows marked paranoid ideation. Speech is rapid and pressured and loud. He appears psychomotor agitated, yelling and pacing the unit and pointing at staff members. Insight is poor. Judgment is poor.

ASSESSMENT/TREATMENT

Mr. Flaherty is a 31 year-old man BIBA on a sec 12 after his mother called the police reporting that pt kicked in her door, was yelling at her and threatening to kill her. Pt's mother states he woke up "angry" today. Pt denies HI/AH/VH, reports passive SI, feeling so overwhelmed "I can't take it anymore".

He presents after a high-risk domestic incident involving threats to kill his mother and a reported knife threat. Even though he minimizes intent and describes it as accidental and sheathed, the reported behavior represents significant acute risk given escalation, impulsivity, agitation, and access to a weapon during conflict. His presentation is characterized by marked irritability, hypervocal speech, severe anxiety, poor sleep, appetite reduction, and impaired behavioral control. He denies current psychotic symptoms and denies current suicidal ideation. He has a history of a suicide attempt, chronic psychosocial stressors, and limited supports.

Agitated, irritable, labile, threatening, demanding to be discharged. He is showing marked irritable and manic symptoms. He insisted on being discharged immediately and was becoming quite agitated but was with effort able to be redirected by staff and accepted olanzapine 10 mg PO and lorazepam 2 mg PO. He denies HI but still appears quite angry with his mother, although at the same time hoping she sill pick him up and bring him home. We will increased olanzapine to 10 mg at bedtime. We will add lorazepam as needed for anxiety and manic symptoms. We will consolidate depakote to 1500 mg daily. We will follow up depakote level tomorrow. We will get collateral as able. We will provides structure and support. We will continue to observe.

Three day letter expires on 2/23/26.

DIAGNOSIS(ES)

Principal Problem:

Bipolar disorder with depression (CMS/HHS/HCC)

Active Problems:

Cannabis use disorder

LEGAL STATUS:

Legal Status Orders

Legal



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02/18/2026 - Admission (Discharged) in Brockton 3 Behavioral Health Inpatient Unit (continued)

Notes (continued)

Section 10 & 11, 3 day letter

FRESH AIR:

Eligible for Fresh Air

I certify that the inpatient psychiatric facility services furnished since the previous certification or recertification were, and continue to be, medically necessary for irritability, lability, erratic behavior, and manic symptoms. Hospital records indicate that the services furnished were either: intensive treatment services, admission and related services necessary for diagnostic study, or equivalent services. I certify that the patient continues to need, on a daily basis, active treatment furnished directly by or requiring the supervision of inpatient psychiatric facility personnel. I estimate 10 days of hospitalization is necessary for proper treatment of the patient. My plans for post-hospital care for this patient are TBD.

Jared K Trout, MD 2/19/2026 9:33 AM

Electronically signed by Jared K Trout, MD at 2/20/2026 11:53 AM

Progress Note - Nursing/Adult by Rebecca Njonjo, RN at 2/19/2026 0632

Patient: 31-year-old White male

Arrival: BIBA at approximately 2315 via stretcher, accompanied by two EMTs. Assisted off stretcher with one assist. Ambulated into intake/admission room with steady gait and balance.

Patient alert and oriented $\times 4$ upon arrival. Initially calm and cooperative with admission process, though intermittently anxious and easily irritable when responding to certain questions. Overall cooperative. Mood appeared stable but affect constricted at times.

Vital Signs:

T 97.5°F, BP 130/87 mmHg, HR 82 bpm, RR 20/min, SpO₂ 98% RA

Weight: 92.5 kg

Height: 180.3 cm

Patient consented to full skin assessment, skin clean, dry, and intact; no open areas noted or reported. Belongings search completed per protocol; no contraband found. Personal clothing secured at bedside.

Reason for Admission

Per report, patient was at home with mother and allegedly threatened her with a knife and damaged a door. Mother contacted 911. Patient was sectioned to the ED for evaluation and subsequently transferred to Brockton Behavioral following stabilization.

Admitting diagnoses reported: PTSD and Bipolar Disorder. Patient endorses depression rated 5/10 and anxiety 10/10. Reports feelings of hopelessness and lack of motivation and frustration due to life stressors. Patient denies SI, HI, AH/VH, and SIB at time of admission.

Admitted by NP Patricia under Sections 11 & 12. Patient placed on 15-minute safety checks. Patient signed a three-

Printed on 4/10/26 10:45 AM

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02/18/2026 - Admission (Discharged) in Brockton 3 Behaviorall Health Inpatient Unit (continued)

Notes (continued)

day letter.

Medical / Substance History

No known drug allergies reported; however, patient unsure and questions possible allergy to Ritalin.
Denies current alcohol use; endorses nicotine and cannabis use.
Denies legal issues.

Admission Course

All intake protocols completed. Patient oriented to unit, room, and rules. Offered food and fluids; tolerated without difficulty. No dysphagia noted. Refused scheduled PRN melatonin for sleep, stating he was tired and preferred to sleep without medication. Patient retired to bed and slept through the night without disturbance. No behavioral incidents or acute distress observed.

Plan:

Continue 15-minute safety checks. Monitor mood, anxiety level, irritability, and potential risk behaviors. Reinforce coping skills and provide emotional support. Continue to assess for safety and clinical stability.

Electronically signed by Rebecca Njonjo, RN at 2/19/2026 6:32 AM

Labs

CBC and differential (Final result)

Specimen Information

ID	Type	Source	Collected By
67621191754742 29	—	—	7522 02/20/26 0952

CBC and differential (Abnormal)

Resulted: 02/20/26 1626, Result status: Final result

Ordering provider: Jared K Trout, MD 02/20/26 0005
Filed by: Interface, Lab In Sunquest 02/20/26 1626
Resulting lab: SUNQUEST

Order status: Completed
Collected by: 7522 02/20/26 0952

Components

Component	Value	Reference Range	Flag	Lab
WBC	9.9	4.0 - 11.0 K/UL	—	SUNQUEST
RBC	5.72	4.40 - 5.60 M/UL	H ^	SUNQUEST
Comment: The results of this assay should be interpreted in the context of the patient's sex-assigned-at-birth and additional relevant clinical and laboratory data.				
Hemoglobin	15.9	13.5 - 17.5 G/DL	—	SUNQUEST
Comment: The results of this assay should be interpreted in the context of the patient's sex-assigned-at-birth and additional relevant clinical and laboratory data.				
Hematocrit	47.7	40.0 - 51.0 %	—	SUNQUEST
Comment: The results of this assay should be interpreted in the context of the patient's sex-assigned-at-birth and additional relevant clinical and laboratory data.				



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02/18/2026 - Admission (Discharged) in Brockton 3 Behaviorall Health Inpatient Unit (continued)

Labs (continued)

MCV	83	80 - 97 FL	—	SUNQUEST
MCH	27.8	27.0 - 33.0 PG	—	SUNQUEST
MCHC	33.3	32.0 - 36.0 G/DL	—	SUNQUEST
Platelet	304	150 - 400 K/UL	—	SUNQUEST
RBC Dist Width	13.4	12.0 - 14.5 %	—	SUNQUEST
NRBC (Percent)	0.0	0.0 /100 WBC	—	SUNQUEST
Absolute NRBC	0.00	0.0 K/UL	—	SUNQUEST
Poly	68	40 - 75 %	—	SUNQUEST
Lymph	23	15 - 54 %	—	SUNQUEST
Mono	7	4 - 13 %	—	SUNQUEST
Eos	1	0 - 7 %	—	SUNQUEST
Baso	1	0 - 1 %	—	SUNQUEST
Absolute Poly	6.8	1.8 - 7.0 K/UL	—	SUNQUEST
Absolute Lymph	2.3	1.1 - 3.5 K/UL	—	SUNQUEST
Absolute Mono	0.7	0.2 - 0.9 K/UL	—	SUNQUEST
Absolute Eos	0.1	0.0 - 0.6 K/UL	—	SUNQUEST
Absolute Baso	0.1	0.0 - 0.1 K/UL	—	SUNQUEST
Immature Granulocytes (Percent)	0	0 - 1 %	—	SUNQUEST
Comment:				
Immature granulocytes represents either metamyelocytes myelocytes, promyelocytes, or any combination thereof.				
Absolute Immature Granulocytes	0.03	0.00 - 0.06 K/UL	—	SUNQUEST

Testing Performed By

Lab - Abbreviation	Name	Director	Address	Valid Date Range
12 - SUNQUEST	SUNQUEST	Reggie Thomasson, MD	BOSTON MEDICAL CENTER LABORATORY CLIA 22D0867404 One Boston Medical Center Place Boston MA 02118	09/25/25 1400 - Present

Comprehensive Metabolic Panel (Final result)

Specimen Information

ID	Type	Source	Collected By
67621191955489 67	—	—	7522 02/20/26 0952

Comprehensive Metabolic Panel (Abnormal)

Resulted: 02/20/26 1647, Result status: Final result

Ordering provider: Jared K Trout, MD 02/20/26 0005
Filed by: Interface, Lab In Sunquest 02/20/26 1647
Resulting lab: SUNQUEST

Order status: Completed
Collected by: 7522 02/20/26 0952

Components

Component	Value	Reference Range	Flag	Lab
Albumin	4.8	3.5 - 5.0 G/DL	—	SUNQUEST
Alkaline Phosphatase, Total	58	25 - 100 U/L	—	SUNQUEST
Comment:				



BMC Yawkey
850 Harrison Ave.
Boston MA02118-4001

Flaherty, Dallas
MRN:3394094
DOB:11/3/1994,Sex:male
SSN:022-78-8206
Visit Date:2/18/2026

02/18/2026 - Admission (Discharged) in Brockton 3 Behaviorall Health Inpatient Unit (continued)

Labs (continued)

The results of this assay should be interpreted in the context of the patient's sex-assigned-at-birth and additional relevant clinical and laboratory data.

ALT(SGPT)	64	9.0 - 67.0 U/L	—	SUNQUEST
AST(SGOT)	46	13 - 39 U/L	H ^	SUNQUEST

Comment: ** PLEASE NOTE NEW REFERENCE RANGE **

Calcium	9.9	8 - 10.5 MG/DL	—	SUNQUEST
CO2	25.0	19 - 28 MMOL/L	—	SUNQUEST

Comment:

Elevated triglyceride levels (>1000 mg/dL) may cause falsely low bicarbonate results. If clinically indicated, a venous blood gas should be ordered to confirm the bicarbonate result.

Chloride	105	98 - 110 MMOL/L	—	SUNQUEST
Glucose	68	70 - 100 MG/DL	L v	SUNQUEST
Potassium	3.5	3.1 - 5.3 MMOL/L	—	SUNQUEST

Comment:

For serum, the lower end of the reference range may be higher by 0.2 to 0.4 mmol/L.

Sodium	141	135 - 145 MMOL/L	—	SUNQUEST
Protein, Total	7.4	6.8 - 8.6 G/DL	—	SUNQUEST
Bilirubin, Total	1.1	0.3 - 1.2 MG/DL	—	SUNQUEST
Urea Nitrogen (BUN)	21	7 - 25 MG/DL	—	SUNQUEST
Creatinine	0.70	0.7 - 1.3 MG/DL	—	SUNQUEST

Comment:

The results of this assay should be interpreted in the context of the patient's sex-assigned-at-birth and additional relevant clinical and laboratory data.

Estimated GFR	>90	>59 mL/min/1.73_m2	—	SUNQUEST
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Comment:

The calculation of eGFR utilizes the 2021 CKD-EPI creatinine equation. eGFR estimates can be inaccurate and may vary from the true level of kidney function. Specific populations in which an eGFR value may be inaccurate or biased include: acute kidney injury, pregnancy, extremes of muscle mass, age greater than 80 years old.

The results of this assay should be interpreted in the context of the patient's sex-assigned-at-birth and additional relevant clinical and laboratory data.

Anion Gap Without Potassium	11	7 - 16	—	SUNQUEST
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Testing Performed By

Lab - Abbreviation	Name	Director	Address	Valid Date Range
12 - SUNQUEST	SUNQUEST	Reggie Thomasson, MD	BOSTON MEDICAL CENTER LABORATORY CLIA 22D0867404 One Boston Medical Center Place Boston MA 02118	09/25/25 1400 - Present

Free T4 (Final result)

Specimen Information

ID	Type	Source	Collected By
67621192156320 81	—	—	7522 02/20/26 0952



BMC Yawkey
850 Harrison Ave.
Boston MA02118-4001

Flaherty, Dallas
MRN:3394094
DOB:11/3/1994,Sex:male
SSN:022-78-8206
Visit Date:2/18/2026

02/18/2026 - Admission (Discharged) in Brockton 3 Behaviorall Health Inpatient Unit (continued)

Labs (continued)

Free T4

Resulted: 02/20/26 1725, Result status: Final result

Ordering provider: Jared K Trout, MD 02/20/26 0005
Filed by: Interface, Lab In Sunquest 02/20/26 1725
Resulting lab: SUNQUEST

Order status: Completed
Collected by: 7522 02/20/26 0952

Components

Component	Value	Reference Range	Flag	Lab
Free T4	1.03	0.6 - 1.8 NG/DL	—	SUNQUEST

Testing Performed By

Lab - Abbreviation	Name	Director	Address	Valid Date Range
12 - SUNQUEST	SUNQUEST	Reggie Thomasson, MD	BOSTON MEDICAL CENTER LABORATORY CLIA 22D0867404 One Boston Medical Center Place Boston MA 02118	09/25/25 1400 - Present

HCV Ab reflex to Confirmatory/Viral load and Genotype (Final result)

Specimen Information

ID	Type	Source	Collected By
67621192206877 8	—	—	7522 02/20/26 0952

HCV Ab reflex to Confirmatory/Viral load and Genotype

Resulted: 02/20/26 1657, Result status: Final result

Ordering provider: Jared K Trout, MD 02/20/26 0005
Filed by: Interface, Lab In Sunquest 02/20/26 1658
Resulting lab: SUNQUEST

Order status: Completed
Collected by: 7522 02/20/26 0952

Components

Component	Value	Reference Range	Flag	Lab
Hepatitis C Antibody	NON- REACTIVE	NON-REACTIVE	—	SUNQUEST

Testing Performed By

Lab - Abbreviation	Name	Director	Address	Valid Date Range
12 - SUNQUEST	SUNQUEST	Reggie Thomasson, MD	BOSTON MEDICAL CENTER LABORATORY CLIA 22D0867404 One Boston Medical Center Place Boston MA 02118	09/25/25 1400 - Present



BMC Yawkey
850 Harrison Ave.
Boston MA02118-4001

Flaherty, Dallas
MRN:3394094
DOB:11/3/1994,Sex:male
SSN:022-78-8206
Visit Date:2/18/2026

02/18/2026 - Admission (Discharged) in Brockton 3 Behaviorall Health Inpatient Unit (continued)

Labs (continued)

Hemoglobin a1c (Final result)

Specimen Information

ID	Type	Source	Collected By
67621192382376 2	—	—	7522 02/20/26 0952

Hemoglobin a1c

Resulted: 02/20/26 1652, Result status: Final result

Ordering provider: Jared K Trout, MD 02/20/26 0005

Order status: Completed

Filed by: Interface, Lab In Sunquest 02/20/26 1652

Collected by: 7522 02/20/26 0952

Resulting lab: SUNQUEST

Components

Component	Value	Reference Range	Flag	Lab
Hemoglobin A1C	4.9	4.0 - 5.6 %	—	SUNQUEST

Comment:

**** PLEASE NOTE NEW REFERENCE RANGE ****

Initial Diagnosis of Diabetes:

Increased risk for diabetes (pre-diabetes): 5.7-6.4%

Diabetes: > or = 6.5%

Patients with Diagnosis of Diabetes:

In patients with diabetes, HbA1c goals should be discussed with a healthcare provider.

The Abbott hemoglobin A1c assay should not be used to diagnose or monitor diabetes in patients with altered red cell lifespan, such as homozygous hemoglobin variants, Hb SC, HbF > 5%, and hemolytic anemia.

Testing Performed By

Lab - Abbreviation	Name	Director	Address	Valid Date Range
12 - SUNQUEST	SUNQUEST	Reggie Thomasson, MD	BOSTON MEDICAL CENTER LABORATORY CLIA 22D0867404 One Boston Medical Center Place Boston MA 02118	09/25/25 1400 - Present

Lipid panel (Final result)

Specimen Information

ID	Type	Source	Collected By
67621192457834 88	—	—	7522 02/20/26 0952

Lipid panel (Abnormal)

Resulted: 02/20/26 1647, Result status: Final result

Ordering provider: Jared K Trout, MD 02/20/26 0005

Order status: Completed

Filed by: Interface, Lab In Sunquest 02/20/26 1647

Collected by: 7522 02/20/26 0952

Resulting lab: SUNQUEST



BMC Yawkey
850 Harrison Ave.
Boston MA02118-4001

Flaherty, Dallas
MRN:3394094
DOB:11/3/1994,Sex:male
SSN:022-78-8206
Visit Date:2/18/2026

02/18/2026 - Admission (Discharged) in Brockton 3 Behaviorall Health Inpatient Unit (continued)

Labs (continued)

Components

Component	Value	Reference Range	Flag	Lab
Cholesterol	164	<200 mg/dL	—	SUNQUEST
Comment: CHOLESTEROL RISK CLASSIFICATION: <200 MG/DL = LOW RISK, 200 to 239 MG/DL = BORDERLINE/HIGH RISK, >239 MG/DL = HIGH RISK.				
Triglyceride	105	40 - 200 MG/DL	—	SUNQUEST
HDL Cholesterol	32	>34 mg/dL	L ▼	SUNQUEST
Comment: HDL CHOLESTEROL RISK CLASSIFICATION: >55 MG/DL = LOW RISK, <35 MG/DL = HIGH RISK.				
LDL Cholesterol, Calculated	111	<130 mg/dL	—	SUNQUEST
Comment: LDL CHOLESTEROL RISK CLASSIFICATION: <130 MG/DL = LOW RISK, 130 to 159 MG/DL = BORDERLINE RISK, >159 MG/DL = HIGH RISK.				

Testing Performed By

Lab - Abbreviation	Name	Director	Address	Valid Date Range
12 - SUNQUEST	SUNQUEST	Reggie Thomasson, MD	BOSTON MEDICAL CENTER LABORATORY CLIA 22D0867404 One Boston Medical Center Place Boston MA 02118	09/25/25 1400 - Present

TSH (Final result)

Specimen Information

ID	Type	Source	Collected By
67621192508204 13	—	—	7522 02/20/26 0952

TSH

Resulted: 02/20/26 1655, Result status: Final result

Ordering provider: Jared K Trout, MD 02/20/26 0005
Filed by: Interface, Lab In Sunquest 02/20/26 1655
Resulting lab: SUNQUEST

Order status: Completed
Collected by: 7522 02/20/26 0952

Components

Component	Value	Reference Range	Flag	Lab
TSH	0.86	0.35 - 4.9 uIU/ML	—	SUNQUEST

Testing Performed By

Lab - Abbreviation	Name	Director	Address	Valid Date Range
12 - SUNQUEST	SUNQUEST	Reggie Thomasson, MD	BOSTON MEDICAL CENTER LABORATORY CLIA	09/25/25 1400 - Present



BMC Yawkey
850 Harrison Ave.
Boston MA02118-4001

Flaherty, Dallas
MRN:3394094
DOB:11/3/1994, Sex: male
SSN:022-78-8206
Visit Date:2/18/2026

02/18/2026 - Admission (Discharged) in Brockton 3 Behaviorall Health Inpatient Unit (continued)

Labs (continued)

22D0867404 One
Boston Medical Center
Place
Boston MA 02118

Valproic acid (depakene) (Final result)

Specimen Information

ID	Type	Source	Collected By
67621192633675 76	—	—	7522 02/20/26 0952

Valproic acid (depakene)

Resulted: 02/20/26 1631, Result status: Final result

Ordering provider: Jared K Trout, MD 02/20/26 0005
Filed by: Interface, Lab In Sunquest 02/20/26 1631
Resulting lab: SUNQUEST

Order status: Completed
Collected by: 7522 02/20/26 0952

Components

Component	Value	Reference Range	Flag	Lab
Valproic Acid (Depakene)	74.9	50 - 100 MCG/ML	—	SUNQUEST

Testing Performed By

Lab - Abbreviation	Name	Director	Address	Valid Date Range
12 - SUNQUEST	SUNQUEST	Reggie Thomasson, MD	BOSTON MEDICAL CENTER LABORATORY CLIA 22D0867404 One Boston Medical Center Place Boston MA 02118	09/25/25 1400 - Present



BMC Yawkey
850 Harrison Ave.
Boston MA02118-4001

Flaherty, Dallas
MRN:3394094
DOB:11/3/1994, Sex: male
SSN:022-78-8206
Visit Date:2/17/2026

02/17/2026 - ED in Boston Medical Center - South Emergency Department

Notes

ED Provider Notes by Gina Zhou Lu, MD at 2/17/2026 1742

History

Chief Complaint

Patient presents with

- Psychiatric Evaluation

BIBA from home on S12 after patient threaten his mother with a knife, kicked and broke the door .Pt has hx of mental illness.

Patient is a 31 y.o. male.

31-year-old male with past psychiatric history, presenting to the emergency room after threatening his mother with a knife. Patient reportedly also kicked and broke down the door. Patient states that he got frustrated because he was stuck in his house without any work to go to. Patient states that he has been attempting to find a job, but has been having difficulty doing that which is furthers his frustration. Patient denies SI, HI, AVH on my evaluation. Patient reports compliance with his medications. Denies further complaints on my evaluation.

Review of Systems

Constitutional: Negative for chills and fever.

Respiratory: Negative for shortness of breath.

Cardiovascular: Negative. Negative for chest pain.

Gastrointestinal: Negative for abdominal distention, abdominal pain, constipation, diarrhea, nausea and vomiting.

Endocrine: Negative.

Genitourinary: Negative for difficulty urinating, dysuria and flank pain.

Musculoskeletal: Negative.

Skin: Negative.

Allergic/Immunologic: Negative.

Neurological: Negative for weakness.

Hematological: Negative.

Physical Exam

BP 143/90 (BP Cuff Location: Left Upper Arm, Patient Position: Lying) | Pulse 90 | Temp 98.1 °F (36.7 °C) (Oral) | Resp 18 | Ht 1.803 m (5' 10.98") | Wt 92.5 kg (203 lb 14.8 oz) | SpO2 97% | BMI 28.45 kg/m²

Physical Exam Constitutional:

General: He is not in acute distress.

Appearance: Normal appearance.

HENT:

Head: Normocephalic and atraumatic.

Eyes:

Extraocular Movements: Extraocular movements intact.



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02/17/2026 - ED in Boston Medical Center - South Emergency Department (continued)

Notes (continued)

Conjunctiva/sclera: Conjunctivae normal.

Pupils: Pupils are equal, round, and reactive to light.

Cardiovascular:

Rate and Rhythm: Normal rate and regular rhythm.

Heart sounds: Normal heart sounds.

Pulmonary:

Effort: Pulmonary effort is normal. No respiratory distress.

Breath sounds: Normal breath sounds. No wheezing, rhonchi or rales.

Abdominal:

General: Abdomen is flat. There is no distension.

Palpations: Abdomen is soft.

Tenderness: There is no abdominal tenderness.

Musculoskeletal:

General: Normal range of motion.

Cervical back: Normal range of motion and neck supple. No rigidity.

Neurological:

General: No focal deficit present.

Mental Status: He is alert and oriented to person, place, and time.

Motor: No abnormal muscle tone.

Skin:

General: Skin is warm and dry.

Psychiatric:

Mood and Affect: Mood normal.

Behavior: Behavior normal.

Vitals reviewed.

Medical Decision Making

31-year-old male with past psychiatric history, presenting to the emergency room after threatening his mother with a knife. Vitals nonactionable. Patient physical exam with heart regular rate and rhythm, lungs grossly clear to auscultation, abdomen soft lax and nontender. Will obtain screening blood work including CBC, CMP, urinalysis, urine drug screen. Patient will require crisis evaluation.

MDM

Final diagnoses:

Bipolar 1 disorder (CMS/HHS/HCC) (Primary)

ED Course as of 02/17/26 2204

Tue Feb 17, 2026

2204 Patient medically cleared. Patient evaluated by crisis and recommended for inpatient bed search. [GL]



BMC Yawkey
850 Harrison Ave.
Boston MA02118-4001

Flaherty, Dallas
MRN:3394094
DOB:11/3/1994,Sex:male
SSN:022-78-8206
Visit Date:2/17/2026

02/17/2026 - ED in Boston Medical Center - South Emergency Department (continued)

Notes (continued)

ED Course User Index

[GL] Gina Zhou Lu, MD

Electronically signed by Gina Zhou Lu, MD at 2/17/2026 10:04 PM

Labs

Comprehensive Metabolic Panel (Final result)

Specimen Information

ID	Type	Source	Collected By
67618835182249 98	—	—	8053 02/17/26 2013

Comprehensive Metabolic Panel (Abnormal)

Resulted: 02/17/26 2041, Result status: Final result

Ordering provider: Gina Zhou Lu, MD 02/17/26 1811

Order status: Completed

Filed by: Interface, Lab In Sunquest 02/17/26 2041

Collected by: 8053 02/17/26 2013

Resulting lab: BMC SOUTH

Components

Component	Value	Reference Range	Flag	Lab
Albumin	5.2	3.5 - 5.0 G/DL	H ^	BMC SOUTH
Alkaline Phosphatase, Total	68	25 - 100 U/L	—	BMC SOUTH
Comment: The results of this assay should be interpreted in the context of the patient's sex-assigned-at-birth and additional relevant clinical and laboratory data.				
ALT(SGPT)	63	9.0 - 67.0 U/L	—	BMC SOUTH
AST(SGOT)	33	13 - 39 U/L	—	BMC SOUTH
Comment: ** PLEASE NOTE NEW REFERENCE RANGE **				
Calcium	9.7	8 - 10.5 MG/DL	—	BMC SOUTH
CO2	25.0	19 - 28 MMOL/L	—	BMC SOUTH
Comment: Elevated triglyceride levels (>1000 mg/dL) may cause falsely low bicarbonate results. If clinically indicated, a venous blood gas should be ordered to confirm the bicarbonate result.				
Chloride	108	98 - 110 MMOL/L	—	BMC SOUTH
Glucose	76	70 - 100 MG/DL	—	BMC SOUTH
Potassium	3.5	3.1 - 5.3 MMOL/L	—	BMC SOUTH
Comment: For serum, the lower end of the reference range may be higher by 0.2 to 0.4 mmol/L.				
Sodium	146	135 - 145 MMOL/L	H ^	BMC SOUTH
Protein, Total	7.6	6.8 - 8.6 G/DL	—	BMC SOUTH
Bilirubin, Total	0.3	0.3 - 1.2 MG/DL	—	BMC SOUTH
Urea Nitrogen (BUN)	9	7 - 25 MG/DL	—	BMC SOUTH
Creatinine	0.61	0.7 - 1.3 MG/DL	L v	BMC SOUTH
Comment:				



BMC Yawkey
850 Harrison Ave.
Boston MA02118-4001

Flaherty, Dallas
MRN:3394094
DOB:11/3/1994,Sex:male
SSN:022-78-8206
Visit Date:2/17/2026

02/17/2026 - ED in Boston Medical Center - South Emergency Department (continued)

Labs (continued)

The results of this assay should be interpreted in the context of the patient's sex-assigned-at-birth and additional relevant clinical and laboratory data.

Estimated GFR	>90	>59 mL/min/1.73_m2	—	BMC SOUTH
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Comment:

The calculation of eGFR utilizes the 2021 CKD-EPI creatinine equation. eGFR estimates can be inaccurate and may vary from the true level of kidney function. Specific populations in which an eGFR value may be inaccurate or biased include: acute kidney injury, pregnancy, extremes of muscle mass, age greater than 80 years old.

The results of this assay should be interpreted in the context of the patient's sex-assigned-at-birth and additional relevant clinical and laboratory data.

Anion Gap Without Potassium	13	7 - 16	—	BMC SOUTH
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Testing Performed By

Lab - Abbreviation	Name	Director	Address	Valid Date Range
1213 - BMC SOUTH	BMC SOUTH	Lori Adcock, MD	BOSTON MEDICAL CENTER LABORATORY CLIA 22D0080440 235 North Pearl Street Brockton MA 02301	10/22/25 0940 - 03/26/26 0922

CBC and differential (Final result)

Specimen Information

ID	Type	Source	Collected By
67618835192387 68	—	—	8053 02/17/26 2013

CBC and differential (Abnormal)

Resulted: 02/17/26 2028, Result status: Final result

Ordering provider: Gina Zhou Lu, MD 02/17/26 1811

Order status: Completed

Filed by: Interface, Lab In Sunquest 02/17/26 2028

Collected by: 8053 02/17/26 2013

Resulting lab: BMC SOUTH

Components

Component	Value	Reference Range	Flag	Lab
WBC	11.2	4.0 - 11.0 K/UL	H ^	BMC SOUTH
RBC	5.67	4.40 - 5.60 M/UL	H ^	BMC SOUTH
Comment: The results of this assay should be interpreted in the context of the patient's sex-assigned-at-birth and additional relevant clinical and laboratory data.				
Hemoglobin	15.8	13.5 - 17.5 G/DL	—	BMC SOUTH
Comment: The results of this assay should be interpreted in the context of the patient's sex-assigned-at-birth and additional relevant clinical and laboratory data.				
Hematocrit	46.5	40.0 - 51.0 %	—	BMC SOUTH
Comment: The results of this assay should be interpreted in the context of the patient's sex-assigned-at-birth and additional relevant clinical and laboratory data.				
MCV	82	80 - 97 FL	—	BMC SOUTH



BMC Yawkey
850 Harrison Ave.
Boston MA02118-4001

Flaherty, Dallas
MRN:3394094
DOB:11/3/1994,Sex:male
SSN:022-78-8206
Visit Date:2/17/2026

02/17/2026 - ED in Boston Medical Center - South Emergency Department (continued)

Labs (continued)

MCH	27.9	27.0 - 33.0 PG	—	BMC SOUTH
MCHC	34.0	32.0 - 36.0 G/DL	—	BMC SOUTH
Platelet	309	150 - 400 K/UL	—	BMC SOUTH
RBC Dist Width	13.5	12.0 - 14.5 %	—	BMC SOUTH
NRBC (Percent)	0.0	0.0 /100 WBC	—	BMC SOUTH
Absolute NRBC	0.00	0.0 K/UL	—	BMC SOUTH
Poly	68	40 - 75 %	—	BMC SOUTH
Lymph	22	15 - 54 %	—	BMC SOUTH
Mono	9	4 - 13 %	—	BMC SOUTH
Eos	1	0 - 7 %	—	BMC SOUTH
Baso	0	0 - 1 %	—	BMC SOUTH
Absolute Poly	7.6	1.8 - 7.0 K/UL	H ^	BMC SOUTH
Absolute Lymph	2.4	1.1 - 3.5 K/UL	—	BMC SOUTH
Absolute Mono	1.0	0.2 - 0.9 K/UL	H ^	BMC SOUTH
Absolute Eos	0.1	0.0 - 0.6 K/UL	—	BMC SOUTH
Absolute Baso	0.1	0.0 - 0.1 K/UL	—	BMC SOUTH
Immature Granulocytes (Percent)	0	0 - 1 %	—	BMC SOUTH
Comment: Immature granulocytes represents either metamyelocytes myelocytes, promyelocytes, or any combination thereof.				
Absolute Immature Granulocytes	0.04	0.00 - 0.06 K/UL	—	BMC SOUTH

Testing Performed By

Lab - Abbreviation	Name	Director	Address	Valid Date Range
1213 - BMC SOUTH	BMC SOUTH	Lori Adcock, MD	BOSTON MEDICAL CENTER LABORATORY CLIA 22D0080440 235 North Pearl Street Brockton MA 02301	10/22/25 0940 - 03/26/26 0922

Urine toxicology screen (Final result)

Specimen Information

ID	Type	Source	Collected By
67619151663933 2	—	—	Joyce Kamau, RN 02/17/26 1925

Urine toxicology screen

Resulted: 02/17/26 2004, Result status: Final result

Ordering provider: Gina Zhou Lu, MD 02/17/26 1811
Filed by: Interface, Lab In Sunquest 02/17/26 2004
Resulting lab: BMC SOUTH

Order status: Completed
Collected by: Joyce Kamau, RN 02/17/26 1925

Components

Component	Value	Reference Range	Flag	Lab
Amphetamine, Urine	NEG	NEG	—	BMC SOUTH
Comment: THIS IS A SCREENING ASSAY ONLY AND THE RESULTS ARE REPORTED AS PRESUMPTIVE POSITIVE OR NEGATIVE, USING A CUTOFF CONCENTRATION OF 1000 NG/ML. RESULTS ARE TO BE USED FOR CLINICAL EVALUATION ONLY. CONFIRMATION TESTING WAS NOT PERFORMED.				



BMC Yawkey
850 Harrison Ave.
Boston MA02118-4001

Flaherty, Dallas
MRN:3394094
DOB:11/3/1994, Sex: male
SSN:022-78-8206
Visit Date:2/17/2026

02/17/2026 - ED in Boston Medical Center - South Emergency Department (continued)

Labs (continued)

Barbiturate, Urine	NEG	NEG	—	BMC SOUTH
Comment: THIS IS A SCREENING ASSAY ONLY AND RESULTS ARE REPORTED AS PRESUMPTIVE POSITIVE OR NEGATIVE, USING A CUTOFF CONCENTRATION OF 200 NG/ML. RESULTS ARE TO BE USED FOR CLINICAL EVALUATION ONLY. CONFIRMATION TESTING WAS NOT PERFORMED.				
Cocaine, Urine	NEG	NEG	—	BMC SOUTH
Comment: THIS IS A SCREENING ASSAY ONLY AND RESULTS ARE REPORTED AS PRESUMPTIVE POSITIVE OR NEGATIVE, USING A CUTOFF CONCENTRATION OF 300 NG/ML. RESULTS ARE TO BE USED FOR CLINICAL EVALUATION ONLY. CONFIRMATION TESTING WAS NOT PERFORMED.				
Opiate, Urine	NEG	NEG	—	BMC SOUTH
Comment: THIS IS A SCREENING ASSAY ONLY AND RESULTS ARE REPORTED AS PRESUMPTIVE POSITIVE OR NEGATIVE, USING A CUTOFF CONCENTRATION OF 300 NG/ML. RESULTS ARE TO BE USED FOR CLINICAL EVALUATION ONLY. CONFIRMATION TESTING WAS NOT PERFORMED.				
Benzodiazepine, Urine	NEG	NEG	—	BMC SOUTH
Comment: THIS IS A SCREENING ASSAY ONLY AND RESULTS ARE REPORTED AS PRESUMPTIVE POSITIVE OR NEGATIVE, USING A CUTOFF CONCENTRATION OF 200 NG/ML. RESULTS ARE TO BE USED FOR CLINICAL EVALUATION ONLY. CONFIRMATION TESTING WAS NOT PERFORMED.				

Testing Performed By

Lab - Abbreviation	Name	Director	Address	Valid Date Range
1213 - BMC SOUTH	BMC SOUTH	Lori Adcock, MD	BOSTON MEDICAL CENTER LABORATORY CLIA 22D0080440 235 North Pearl Street Brockton MA 02301	10/22/25 0940 - 03/26/26 0922

Expanded opioid panel, urine (Final result)

Specimen Information

ID	Type	Source	Collected By
676191516641773	—	—	Joyce Kamau, RN 02/17/26 1925

Expanded opioid panel, urine

Resulted: 02/17/26 2004, Result status: Final result

Ordering provider: Gina Zhou Lu, MD 02/17/26 1811
Filed by: Interface, Lab In Sunquest 02/17/26 2004
Resulting lab: BMC SOUTH

Order status: Completed
Collected by: Joyce Kamau, RN 02/17/26 1925

Components

Component	Value	Reference Range	Flag	Lab
Buprenorphine, Urine	NEG	NEG	—	BMC SOUTH
Comment: THIS IS A SCREENING ASSAY ONLY AND RESULTS ARE REPORTED AS PRESUMPTIVE POSITIVE OR NEGATIVE, USING A CUTOFF CONCENTRATION OF 5 NG/ML. RESULTS ARE TO BE USED FOR CLINICAL EVALUATION ONLY. CONFIRMATION TESTING WAS NOT PERFORMED.				
Oxycodone, Urine	NEG	NEG	—	BMC SOUTH
Comment:				



BMC Yawkey
850 Harrison Ave.
Boston MA02118-4001

Flaherty, Dallas
MRN:3394094
DOB:11/3/1994,Sex:male
SSN:022-78-8206
Visit Date:2/17/2026

02/17/2026 - ED in Boston Medical Center - South Emergency Department (continued)

Labs (continued)

THIS IS A SCREENING ASSAY ONLY AND RESULTS ARE REPORTED AS PRESUMPTIVE POSITIVE OR NEGATIVE, USING A CUTOFF CONCENTRATION OF 100 NG/ML. RESULTS ARE TO BE USED FOR CLINICAL EVALUATION ONLY. CONFIRMATION TESTING WAS NOT PERFORMED.

Methadone, Urine	NEG	NEG	—	BMC SOUTH
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Comment:

THIS IS A SCREENING ASSAY ONLY AND RESULTS ARE REPORTED AS PRESUMPTIVE POSITIVE OR NEGATIVE, USING A CUTOFF CONCENTRATION OF 300 NG/ML. RESULTS ARE TO BE USED FOR CLINICAL EVALUATION ONLY. CONFIRMATION TESTING WAS NOT PERFORMED.

Fentanyl Presumptive Screen, Urine	NEG	NEG	—	BMC SOUTH
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Comment:

This is a screening assay only and results are reported as presumptive positive or negative, using a cutoff concentration of 1 ng/mL. Results are to be used for clinical evaluation only. Positive samples will be sent out for confirmation.

Testing Performed By

Lab - Abbreviation	Name	Director	Address	Valid Date Range
1213 - BMC SOUTH	BMC SOUTH	Lori Adcock, MD	BOSTON MEDICAL CENTER LABORATORY CLIA 22D0080440 235 North Pearl Street Brockton MA 02301	10/22/25 0940 - 03/26/26 0922

THC (marijuana), urine (Final result)

Specimen Information

ID	Type	Source	Collected By
67619151914477 2	—	—	Joyce Kamau, RN 02/17/26 1925

THC (marijuana), urine (Abnormal)

Resulted: 02/17/26 2004, Result status: Final result

Ordering provider: Gina Zhou Lu, MD 02/17/26 1811

Order status: Completed

Filed by: Interface, Lab In Sunquest 02/17/26 2004

Collected by: Joyce Kamau, RN 02/17/26 1925

Resulting lab: BMC SOUTH

Components

Component	Value	Reference Range	Flag	Lab
Thc (Marijuana), Urine	POS	NEG	A !	BMC SOUTH

Comment:

THIS IS A SCREENING ASSAY ONLY AND RESULTS ARE REPORTED AS PRESUMPTIVE POSITIVE OR NEGATIVE, USING A CUTOFF CONCENTRATION OF 50 NG/ML. RESULTS ARE TO BE USED FOR CLINICAL EVALUATION ONLY. CONFIRMATION TESTING WAS NOT PERFORMED.

Testing Performed By

Lab - Abbreviation	Name	Director	Address	Valid Date Range
1213 - BMC SOUTH	BMC SOUTH	Lori Adcock, MD	BOSTON MEDICAL CENTER LABORATORY CLIA 22D0080440 235 North	10/22/25 0940 - 03/26/26 0922



BMC Yawkey
850 Harrison Ave.
Boston MA02118-4001

Flaherty, Dallas
MRN:3394094
DOB:11/3/1994, Sex: male
SSN:022-78-8206
Visit Date:2/17/2026

02/17/2026 - ED in Boston Medical Center - South Emergency Department (continued)

Labs (continued)

Pearl Street
Brockton MA 02301